



Internal Medicine Residency Program

Methodist Healthcare Graduate Medical Education · San Antonio, Texas

# Annual Program Evaluation

**Bravein Amalakuhan, MD · Program Director, Internal Medicine**

Presented to: Program Evaluation Committee (PEC) & Core Faculty · April 2026

Accreditation Status: Initial Accreditation with Warning (Effective 09/12/2025)

6

Citations

6 of 6

Completed

Sept

2026 Site Visit

## A MESSAGE TO THE TEAM

“

A program built on **structure, evidence, and continuous improvement** —

”

measured against national benchmarks, refined by resident and faculty feedback, and rebuilt around what the data tells us works.

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METHODIST HEALTHCARE SAN ANTONIO • INTERNAL MEDICINE RESIDENCY • SELF-STUDY 2026

*Preparing the seeds for decades to come.*

## Agenda

| ACGME & Program Requirements                                                                                                                                                                | Resident Performance                                                                                                                                                    | Program Quality                                                                                                                                                                                     | Faculty Performance                                                                                           | Forward Planning                                                                               |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------|
| <ul style="list-style-type: none"><li>● Accreditation Status &amp; Citations</li><li>● 2025–26 Action Plan Review</li><li>● SWOT Analysis (2025–26)</li><li>● ABIM Board Planning</li></ul> | <ul style="list-style-type: none"><li>● ITE Results (2024 &amp; 2025 Trend)</li><li>● Milestone Achievement</li><li>● Scholarly Activity</li><li>● Work Hours</li></ul> | <ul style="list-style-type: none"><li>● ACGME Survey Results (2025)</li><li>● Wellness</li><li>● Recruitment</li><li>● Health Equity &amp; Inclusion</li><li>● Didactics &amp; Curriculum</li></ul> | <ul style="list-style-type: none"><li>● Scholarly Activity</li><li>● Faculty Development Activities</li></ul> | <ul style="list-style-type: none"><li>● SWOT (2026–27)</li><li>● 2026–27 Action Plan</li></ul> |

*Current as of April 2026*

## Accountability in Action: AY 2025-2026 APE

### FROM THE PROGRAM DIRECTOR

- **ACGME site visit: April 30, 2025** — conducted prior to current leadership tenure.
- **New PD appointed June 2025** following transition of prior PD and Program Administrator.
- **Gaps identified immediately:** curriculum structure, ABIM alignment, milestone-anchored evaluations, transitions of care (no night float), progressive autonomy framework.

#### *We did not wait for results to begin the work.*

- **June–September 2025 rebuild:** 13.5-hr/week ABIM-aligned curriculum; night float (4/2025) + I-PASS handoffs; real-time evaluation system; 17-page autonomy policy; 18 named faculty leadership roles; faculty development library build.
- **Leadership team expanded:** 2 new Program Administrators + 2 new APDs onboarded (3 APDs total).
- **Site visit results received September 30, 2025:** Initial Accreditation with Warning, 6 citations — each matched a gap already being addressed.
- **All 6 citations → Completed by end of December 2025** — within 90 days of notification.

### — Bravein Amalakuhan, MD

#### *A record, not a defense.*

- Every system described is operational.
- Every person named has a defined role and knows it.
- Every claim is verifiable in New Innovations, ADS, or program documentation.
- **Quality & Patient Safety Curriculum:** monthly RCA sessions, resident-led QI projects, redesigned patient safety didactic framework — a core educational pillar.

The citations identified what this program needed to optimize.

*We used them as a blueprint — one we had already started drawing.*

# ACGME Accreditation & Citation Response

- ▶ Accreditation Status: Initial Accreditation with Warning — 6 Citations Fully Remediated
- ▶ Citation #1: Didactic Curriculum Rebuild — 13.5 hrs/wk, +238%, ABIM-aligned
- ▶ Citation #2: 18-Faculty Directorship Structure Creation + Attendance
- ▶ Citation #3: 17-page Progressive Autonomy Policy
- ▶ Citation #4: Evaluation Completion 50% → > 90%
- ▶ Citation #5: Night Float + I-PASS Handoff System
- ▶ Citation #6: Patient Safety & QI Culture Overhaul
- ▶ And More!

## Accreditation Status

*Letter of Notification received from Internal Medicine Review Committee | Effective September 2025 · New Program Director appointed June 2025 — all corrective actions initiated within 90 days of appointment*

### CURRENT STATUS

**Initial Accreditation  
with Warning**

*Effective: September 2025*

### NEXT SITE VISIT

**September 1, 2026  
(Approximate)**

*Monitor via ADS Important Dates dashboard*

### TOTAL CITATIONS

**6 Citations**

*All 6 citations fully remediated · COMPLETED December 2025*

### CITATIONS — SUMMARY

1

Inadequate Didactic Curriculum —  
unstructured, no ABIM alignment

2

Inadequate Faculty Responsibilities —  
sporadic attendance, no advance  
scheduling

3

Inadequate Progressive Responsibility —  
no formal autonomy policy or framework

4

Inadequate Feedback & Evaluations —  
no timely, milestone-anchored written  
evals

5

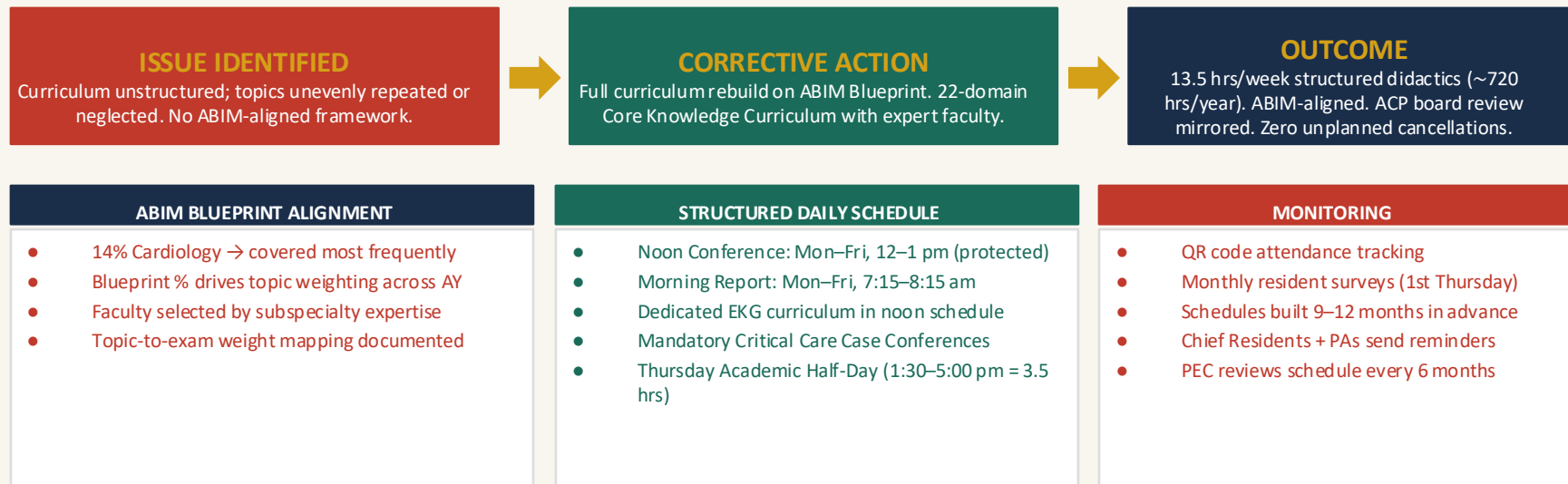
Inadequate Transitions of Care —  
no night float, no standardized handoff  
process

6

Inadequate Patient Safety Culture —  
insufficient structured QI and RCA  
participation

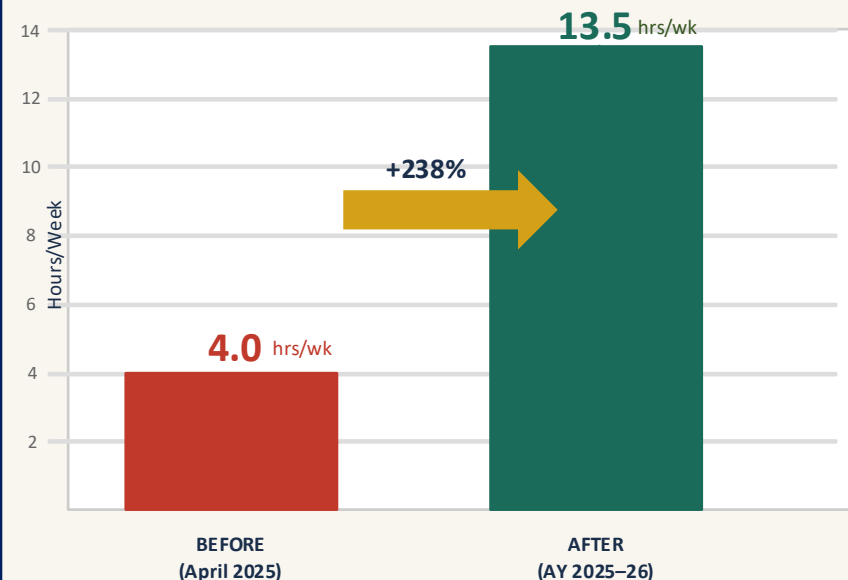
# Citation #1 — Inadequate Didactic Curriculum

ACGME CPR 4.11.g / 4.11.g.1 | Status:  COMPLETED | Response filed February 2026



**SITE VISITOR READINESS (Note: All changes initiated within 90 days of new PD appointment, June 2025):** Didactic schedules built 6–12 months in advance and available for review. Attendance logs in New Innovations. MKSAP distributed to all residents. Morning Report and Noon Conference attendance tracked via QR code. Monthly resident feedback surveys on file. Program now delivers **13.5 structured didactic hours per week (~720 hrs/year) — curriculum mirrors ACP board review programs** vs. pre-citation baseline.

## Didactic Hours — Before vs. After: The Impact of Restructuring



| ACTIVITY                                | BEFORE             | NOW                |
|-----------------------------------------|--------------------|--------------------|
| Thursday Conference (moved to 1:30–5pm) | 4.0 hrs/wk         | 3.5 hrs/wk         |
| Morning Report (Mon–Fri)                | —                  | 5.0 hrs/wk         |
| Noon Conference (Mon–Fri)               | —                  | 5.0 hrs/wk         |
| Monthly RCA Session                     | —                  | 30 min/mo          |
| Monthly Board Review (Dr.G)             | —                  | 1 hr/mo            |
| <b>TOTAL WEEKLY</b>                     | <b>4.0 hrs/wk</b>  | <b>13.5 hrs/wk</b> |
| <b>ANNUALIZED</b>                       | <b>~130 hrs/yr</b> | <b>~720 hrs/yr</b> |

**TIMELINE:** April 2025— ACGME site visit (Thursday-only curriculum, 4.0 hrs/wk) · June 2025 — New PD appointed · July 2025 — Full restructured curriculum launched ·  
 AY 2025–26 — 13.5 hrs/wk · ~720 hrs/year · +238% increase



## ABIM Blueprint Alignment — “Three-Layer Curriculum Architecture”

*18 of 18 ABIM Blueprint categories covered · 13.5 hr/week core didactics · Structured across three reinforcing layers*

### LAYER 1 · CORE MODULES

69

#### ACP- ALIGNED MODULES

~345 discrete topics across all 18 ABIM blueprint categories, mirroring ACP Board Review structure.

- Cardiology (5), Endo (7), GI (7)
- GIM (13), Heme (5), ID (7)
- Nephro (5), Neuro (4), Onc (4)
- PCCM (7), Rheum (5)



### LAYER 2 · SUBSPECIALTY

~120

#### HRS/YR SUPPLEMENTAL

Thursday afternoon subspecialty sessions + targeted advanced lectures reinforcing high-weight areas.

- Weekly Critical Care Conference
- 3 CVICU + CV Physiology + EKG
- Pulm Fibrosis + Pulm HTN advanced
- 2 Addiction Medicine + Depression
- Dedicated OB/Gyn + POCUS monthly



### LAYER 3 · BOARD REVIEW

18/18

#### CATEGORIES COVERED

Monthly HCA IM Board Review rotates every ABIM category + monthly Gidwani sessions.

- Monthly HCA Blueprint Board Review
- Monthly Dr. Gidwani Board Review (1 hr)
- Monthly Dr. Patel Board Review (1hr)
- Daily morning report cases
- ACP Medical Jeopardy Sessions



## Blueprint Coverage Matrix — 18 Categories · Curriculum vs. ABIM Weight

| ABIM Category       | ABIM BP % | Curric % | Status |
|---------------------|-----------|----------|--------|
| Cardiovascular      | 14%       | 14%      | ✓      |
| Endo/Diabetes/Metab | 9%        | 9.6%     | ✓      |
| Gastroenterology    | 9%        | 11.0%    | ✓      |
| Infectious Disease  | 9%        | 10.4%    | ✓      |
| Pulmonary Disease   | 9%        | 12%      | ✓      |
| Rheum/Orthopedics   | 9%        | 7.8%     | ✓      |
| Hematology          | 6%        | 7.8%     | ✓      |
| Nephrology/Urology  | 6%        | 7.0%     | ✓      |
| Medical Oncology    | 6%        | 6%       | ✓      |

| ABIM Category        | ABIM BP % | Curric % | Status |
|----------------------|-----------|----------|--------|
| Neurology            | 4%        | 5.8%     | ✓      |
| Psychiatry           | 4%        | 4%       | ✓      |
| Dermatology          | 3%        | 4%       | ✓      |
| Obstetrics/Gyn       | 3%        | 3%       | ✓      |
| Geriatric Syndromes  | 3%        | 5%       | ✓      |
| Allergy/Immunology   | 2%        | 2%       | ✓      |
| Misc (Epi/Ethics/QI) | 2%        | 3%       | ✓      |
| Ophthalmology        | 1%        | 1%       | ✓      |
| Oto/Dental           | 1%        | 1%       | ✓      |

*All 18 of 18 ABIM blueprint categories met or exceeded. · Zero unaddressed content gaps.*

## Supplemental Blueprint-Reinforcing Content for HIGH-YIELD AREAS — By Category

### CARDIOVASCULAR · 14%

3 CVICU lectures · CV Physiology · Longitudinal EKG curriculum · Monthly CVD lectures (Prasad, Triana, Wright) · ACLS Sim I & II

### PULMONARY · 12%

Pulmonary Fibrosis · Pulmonary HTN · Weekly Pulm/CCM Conf · Monthly Pulm/ICU lectures (Abedi, Gidwani) · POCUS series (Vazquez)

### GASTROENTEROLOGY · 11%

Monthly GI lectures · Hepatology & Cirrhosis cases · GI Bleed protocols · IBD review · GI emergencies in Morning Report

### INFECTIOUS DISEASE · 10%

Monthly ID lectures · Antibiotic stewardship · HIV/OI cases · Sepsis bundle · Travel medicine · ID cases daily in Morning Report

### ENDOCRINOLOGY · 10%

Diabetes management · Thyroid disorders · Adrenal · Pituitary · DKA/HHS · Monthly endo lectures · Board-style case review

### RHEUMATOLOGY · 8%

Autoimmune disease review · RA, SLE, vasculitis · Crystal arthropathy · Rheum in Morning Report · MSK-focused board review

### HEMATOLOGY / ONCOLOGY · 14%

Heme malignancies · Anemia workup · Coag disorders · Oncologic emergencies · Tumor Board · Monthly Heme-Onc lectures

### NEPHROLOGY · 7%

AKI/CKD review · Electrolyte disorders · Acid-base · Dialysis · Renal physiology · Monthly Nephro lectures

### CROSS-CUTTING · DAILY

Daily Morning Report · Noon Conference M-F (protected) · Thursday didactics · QI/RCA · Journal Club · Bioethics · M&M · Board Review Sessions · Medical Jeopardy Sessions

*High-yield ABIM blueprint domains reinforced through structured longitudinal lecture series*

## Citation #2 — Inadequate Responsibilities of Faculty

Faculty conference participation & educational engagement | Status:  COMPLETED | Response filed February 2026

### BEFORE — STATE AT SITE VISIT

- Faculty attendance at conferences was sporadic
- Conferences occasionally canceled with little notice
- Less structured faculty roles



### AFTER — CURRENT STATE

- Faculty attendance now consistently near 100%
- Zero unplanned conference cancellations
- Schedules built 9–12 months in advance
- Contingency coverage pre-arranged for every slot
- New PD (June 2025) restructured entire faculty into named Leadership Domains — 18 faculty directors with defined roles

### FACULTY ENGAGEMENT PROCESS FLOW

Schedule Built  
9–12 Months  
In Advance

PA Assigns  
Faculty to  
Each Session

Chief Resident  
Reminder  
1 Week Prior

PA Secondary  
Reminder  
48 hrs Prior

Session  
Delivered +  
Attendance Logged

Monthly Resident  
Survey — Quality  
Assessment

100%

Faculty Attendance  
(Current)

0

Unplanned  
Cancellations

9–12mo

Schedule Built  
In Advance

MONITORING: QR code attendance • Monthly resident surveys (1st Thursday) • PD/APD review of completion rates • Chief Resident weekly engagement review

## Faculty Leadership Restructuring — 18 Named Directors, Defined Roles & Accountability

New PD appointed June 2025 | 18 named faculty directors across defined leadership domains | Matrix-style governance framework | Directly addresses Citation #2

**CITATION #2 ROOT CAUSE:** Faculty participation inconsistent — no defined roles or accountability. **SOLUTION:** Within 90 days of new PD (June 2025), Faculty Leadership Domain framework implemented — 18 named directorships with defined responsibilities & accountability.



Each domain leader addresses and owns major ACGME citations in direct partnership with the Program Director — driving gap closure, programmatic accountability, and continuous quality improvement of program aims.

# Faculty Engagement Point System — The Mechanism

*A transparent, auditable framework for faculty teaching, scholarship, and program citizenship · Modeled on published precedents — every weight benchmarked*

*When ACGME asks "how do you know your faculty are engaged?" — narrative is not enough. This system converts engagement into data. Six domains. 100 points. Four tiers. Objective. Auditable.*

**SIX DOMAINS · PER-DOMAIN MAXIMUMS · 100 PTS / YR · 60 = THRESHOLD**

|          |                                                                                                                 |               |
|----------|-----------------------------------------------------------------------------------------------------------------|---------------|
| <b>1</b> | <b>Teaching &amp; Didactics</b><br><i>Noon conf · morning report · journal club · simulation · grand rounds</i> | <b>MAX 30</b> |
| <b>2</b> | <b>Clinical Supervision</b><br><i>Ward/ICU attending · mentorship · clinic precepting</i>                       | <b>MAX 25</b> |
| <b>3</b> | <b>CCC / PEC / Committees</b><br><i>Evaluation · milestones · governance · recruitment</i>                      | <b>MAX 15</b> |
| <b>4</b> | <b>Scholarly Activity</b><br><i>Publications · posters · chapters · grants · reviewing</i>                      | <b>MAX 15</b> |
| <b>5</b> | <b>Quality / Program Improvement</b><br><i>QI mentoring · M&amp;M · APE working groups · curriculum design</i>  | <b>MAX 10</b> |
| <b>6</b> | <b>Faculty Development</b><br><i>FD sessions · external conferences · certifications</i>                        | <b>MAX 5</b>  |

**RETROACTIVE BASELINE · PLANNED AY 2025–2026**

## Retroactive Faculty Point Assessment

Each faculty member will receive a baseline score for contributions already made this AY.

## RECOGNITION TIERS

|                                          |                    |
|------------------------------------------|--------------------|
| <b>PLATINUM</b>                          | <b>90+ pts</b>     |
| <i>Master Educator · DIO letter</i>      |                    |
| <b>GOLD</b>                              | <b>75–89 pts</b>   |
| <i>Retreat recognition · award slate</i> |                    |
| <b>SILVER</b>                            | <b>60–74 pts</b>   |
| <i>In good standing · reappointment</i>  |                    |
| <b>BRONZE / AT-RISK</b>                  | <b>&lt; 60 pts</b> |
| <i>Mid-year PD conversation · plan</i>   |                    |

## PUBLISHED PRECEDENTS

|                                     |                                          |
|-------------------------------------|------------------------------------------|
| <b>OHSU EM — Ma 2017</b>            | <i>10-yr longitudinal gold standard</i>  |
| <b>NYU EM — Carmody 2019</b>        | <i>ARVU methodology · 87% compliance</i> |
| <b>Yeh — J Gen Intern Med 1999</b>  | <i>Earliest IM-specific model</i>        |
| <b>Yale Pathology — Morrow 2021</b> | <i>30% teaching cap (we match)</i>       |

*Every weight is benchmarked. None is arbitrary. When ACGME asks "where did this come from?" — we answer with citations.*

# Point System — Governance, Workflow & APE Integration

Monthly logging · quarterly audits · mid-year PD review · annual tier assignment · auto-feeds APE, ADS, and citation response evidence

## DOCUMENTATION & WORKFLOW · HOW POINTS ARE CAPTURED

01

### Monthly Self-Log

Each faculty member logs activities in FacultyHub App. Evidence description + file upload required/email PA.

02

### Quarterly Audit

Program coordinator reconciles logs against MedHub attendance, rotation schedules, procedure logs, PubMed. Discrepancies → PD.

03

### Mid-Year PD Review

January: PD meets with any faculty tracking below 30 pts at mid-year. Individualized plan documented. Filed in faculty folder.

04

### End-of-Year Compile

Totals locked June 30. Feeds APE scholarly-activity section, faculty annual review, ADS scholarly activity table.

05

### Public Reporting

Aggregate (de-identified) distribution shared at annual faculty retreat. Individual totals shared only with faculty member + DIO.

## IMPLEMENTATION TIMELINE · ROLLOUT TO SEP 2026

|            |                   |                                                   |
|------------|-------------------|---------------------------------------------------|
| April 2026 | PEC Approval      | Formal vote · minute-documented                   |
| JUN 2026   | Faculty Town Hall | PD-led · Q&A · Jul 1 launch confirmed             |
| JUL 2026   | AY 26–27 Launch   | MedHub portfolio live · logging begins (PA email) |
| AUG 2026   | Pre-Visit Audit   | Dry-run aggregate report · coaching               |
| SEP 2026   | ACGME Site Visit  | II.B.4 / II.B.5 / IV.D evidence live              |
| JUN 2027   | First Cycle Close | Year-1 tiers · APE/ADS integration                |

## GOVERNANCE & ROLES

|                                                      |                                                                      |
|------------------------------------------------------|----------------------------------------------------------------------|
| <b>Program Director</b><br><i>Dr. Amalakuhan</i>     | Framework · tier assignments · at-risk meetings · GMEC/DIO reporting |
| <b>APDs</b><br><i>Drs. Melo, Sanka, Triana</i>       | Domain-specific review · gap flags · Domain-6 remediation            |
| <b>Program Coordinator</b><br><i>GME Coordinator</i> | MedHub portfolio · quarterly reconciliation · reports                |
| <b>Chief Residents</b><br><i>Drs. Melo, Wright</i>   | Validate teaching attendance · engagement feedback                   |
| <b>DIO</b><br><i>Dr. Justin Williams</i>             | Annual institutional rollup · Platinum recognition letters           |

## Citation #3 — Inadequate Progressive Responsibility

Resident autonomy & graded clinical authority | Status:  COMPLETED | 17-page Policy filed November 2025

### ROOT CAUSES IDENTIFIED

- All residents new to institution simultaneously
- No Senior Residents
- Culture of early consultant engagement predating GME
- No formal autonomy policy



### INTERVENTIONS IMPLEMENTED

- Faculty Development sessions on progressive autonomy — consultants contacted AFTER resident formulates specific question
- Expectation reset: residents initiate full diagnostic workup and management plan independently first
- 17-page Formal Progressive Autonomy Policy — milestone-based, by PGY level and quarter, on New Innov
- PGY-2/3 lead inpatient & ICU teams, run rounds, supervise interns, serve as primary contact for nurse teams

### QUARTER-BY-QUARTER AUTONOMY PROGRESSION (PGY-1)

#### Q1 Jul–Oct

3–5 pts  
Orders: Direct Supervision  
Consult: Supervised



#### Q2 Nov–Feb

10 pts  
Orders: Independent  
Consult: Supervised



#### Post-CCC Mid-Year

Up to 10 (indep.)  
Orders: Full Autonomy  
Consult: Full Autonomy



#### Q3 Mar–Jun

10 pts  
Orders: Independent  
Consult: Independent

### ICU GRADUATED CAP (INTERN)

- Week 1–2 July: 2–3 patients | Week 3–4 July: 3–4 patients
- August: 4–5 → 5–6 patients (biweekly steps)
- September: 7 patients maximum
- All intern orders directly supervised by senior — first 3 months, no exceptions
- Attending on-site 24/7 in ICU

### AMBULATORY + MONITORING

- Clinic: PGY-1 = 2–4 visits/session · PGY-2 = 6/session · PGY-3 = 8/session
- PGY-2/3 lead rounds, supervise interns, run codes
- Lines of authority: PGY-1 → Senior → Chief → APD → PD
- Monthly resident surveys assess autonomy experience
- Objective criteria — not subjective assessment

**RESULT:** Improved resident confidence, leadership engagement & clarity in role expectations · Policy available in New Innovations · Monthly survey monitoring active



# Progressive Autonomy Policy & Procedure — AY 2025–2026

17-page formal policy | Addresses ACGME Citation 4 | Distributed to all residents & faculty | Accompanied by faculty development lecture series

## SUPERVISION FRAMEWORK

**Direct Supervision** Attending physically present with resident

**Indirect Supervision** Attending in-house, immediately available or by phone

**Oversight** Post-hoc review with structured feedback

*PGY-1 residents supervised directly or indirectly at all times*

## PGY-LEVEL PROGRESSION · BY QUARTER

**PGY-1 | Q1 (Jul–Oct)** H&P, data gathering; 3–4 pts; supervised orders & consults

**PGY-1 | Q2 (Nov–Feb)** 10 pts; supervised consultant communication; independent order entry

**PGY-1 | Q3 (Mar–Jun)** 10 pts; independent consults & orders; coaching toward PGY-2

**PGY-2/3 | All Quarters** Oversees all care; supervises interns; runs codes; coaches to independence

## FLOOR — INTERN PATIENT CAP

- First 2 wks July: **3–5 patients** (direct supervision)
- Last 2 wks July: **4–6 patients**
- August (biweekly): **6–8 → 8–10 pts**
- September: **Max 10 patients**

*All intern orders directly supervised by senior — first 3 months. No exceptions. After mid-year CCC: intern may work independently per CCC.*

## ICU — INTERN PATIENT CAP

- First 2 wks July: **2–3 patients**
- Last 2 wks July: **3–4 patients**
- August (biweekly): **4–5 → 5–6 pts**
- September: **7 patients max**

*All intern orders directly supervised by senior for first 3 months. Attendings on-site 24/7 in ICU. No exceptions.*

## AMBULATORY + POLICIES

- Clinic visits/(half day session): PGY-1=2–4 · PGY-2=6 · PGY-3=8
- Code Blue: PGY-1 compressions · PGY-2/3 runs the code
- ICU residents respond to ALL codes — no exceptions

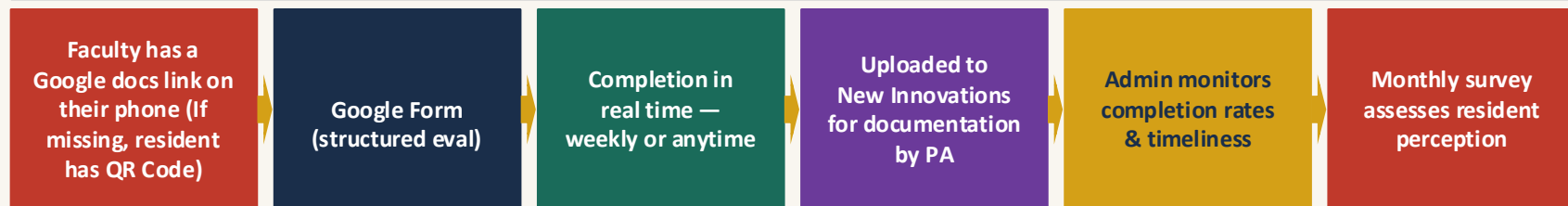
**Lines of Authority:** PGY-1 → Senior → Chief → APD → PD

## Citation #4 — Inadequate Feedback & Evaluations by Faculty

Milestone-based, timely, documented evaluations | Status:  COMPLETED | Completion rate: 50% → > 90% post new Strategy | Link system deployed Fall 2025



### “Online Link” REAL-TIME EVALUATION WORKFLOW



#### FACULTY DEVELOPMENT SESSIONS

- Dedicated sessions on timely, structured, milestone-based feedback
- Importance of documented evaluations vs. verbal feedback only
- Training on Level 2/3/4 behavioral descriptor language
- CCC narrative language standardized across all faculty
- Evaluation redesign — focused, high-value, efficient form

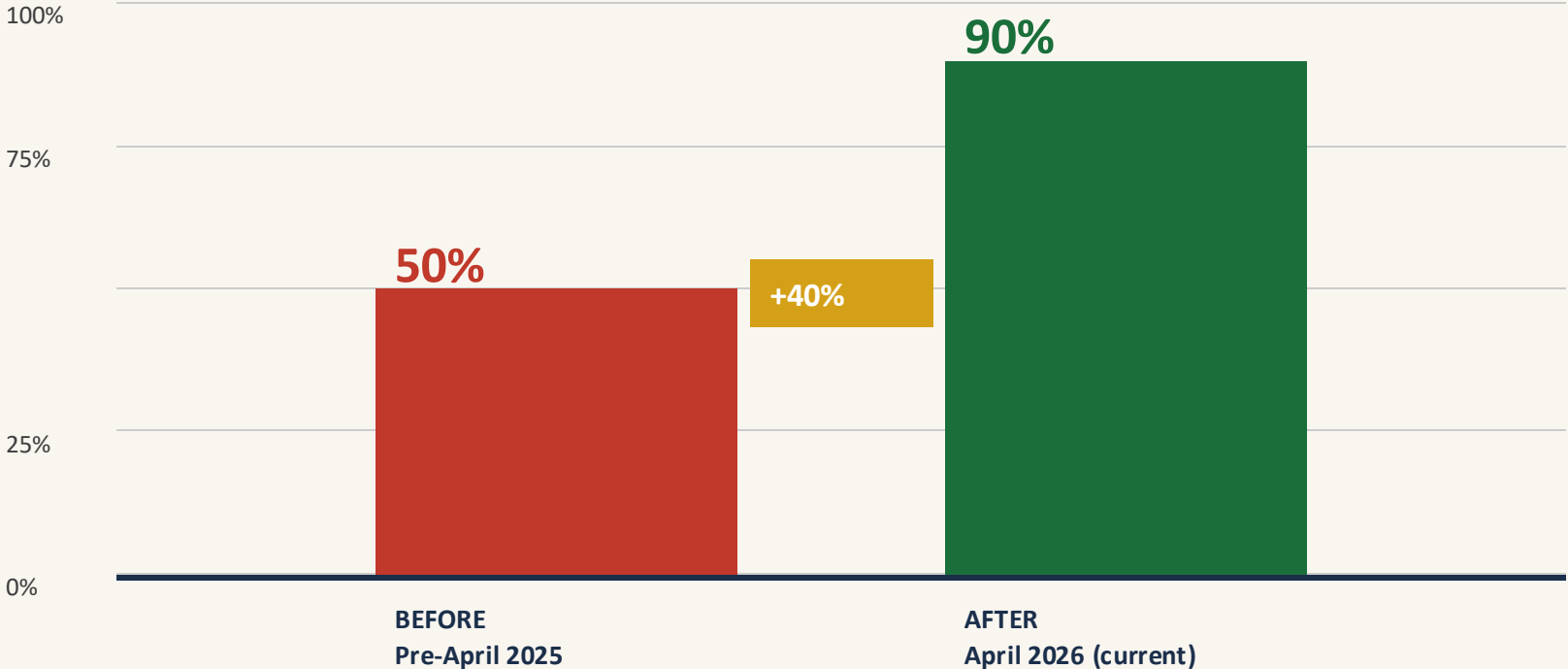
#### MONITORING & COMPLIANCE

- Monthly resident surveys: quality, timeliness, and impact rated
- Program leadership monitors NI evaluation completion rate audits
- Faculty may evaluate weekly or at any rotation point
- More frequent formative feedback — not just end-of-rotation
- Improved timeliness, frequency, and actionability confirmed

**RESULT:** Faculty evaluation completion rate increased from 50% → 90% since new PD appointment (June 2025) · Improved timeliness · Structured, milestone-anchored, documented in New Innovations

# Evaluation Completion Rate — Before vs. After

*Since new Faculty Appointments (June 2025) · measured via New Innovations submissions*

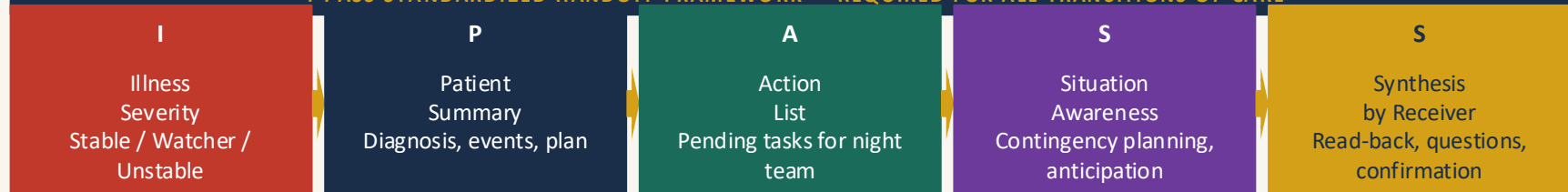


## Citation #5 — Inadequate Transitions of Care

Standardized handoffs, night float, I-PASS | Status:  COMPLETED | Night float operational July 2025

**ROOT CAUSE:** No structured resident-led night float system → no continuity or ownership of transitions between day and night teams. Sign-out was informal, non-standardized, and lacked direct resident-to-resident communication.

### I-PASS STANDARDIZED HANDOFF FRAMEWORK — REQUIRED FOR ALL TRANSITIONS OF CARE



#### NIGHT FLOAT SYSTEM IMPLEMENTED

- Dedicated resident-led night float (4/2025) — daily continuity /accountability
- 7:00 am — Night-to-Day transition (direct resident-to-resident)
- 7:00 pm — Day-to-Night transition (direct resident-to-resident)
- Nocturnal Medicine Service: Dr. Sumeet Chavan, Director
- In-person oversight by Inpatient nocturnist
- WebEx option available
- PD and APDs conduct periodic audits of handoff process

#### SUPERVISION & MONITORING

- Structured sign-out sheet: patient name, room, Dx, attending, status, action items
- Issues requiring follow-up written in detail for Night Float
- Census updated daily, printed for sign-out
- PD/APD monthly check-ins with residents on handoff quality
- Monthly resident surveys assess safety, educational value
- Ongoing observation by attendings and nocturnists
- I-PASS framework training embedded in orientation

**RESULT:** Structured, resident-led, standardized, supervised transitions-of-care process · Promotes patient safety, accountability & continuity · I-PASS now program standard

# Citation #6 — Inadequate Culture of Patient Safety & Teamwork

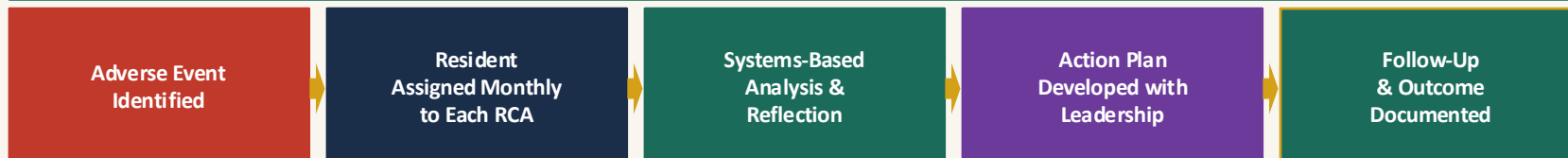
Structured QI, patient safety curriculum & resident RCA participation | Status:  COMPLETED | September 2025 | Nick Mathers + Dr.Abedi QI curriculum lead

**ROOT CAUSE:** Resident participation in patient safety was insufficient — less structured RCA process, limited QI curriculum breadth, no guaranteed all-resident participation. FULLY REMEDIATED September 2025.

## 6 ANNUAL STRUCTURED PATIENT SAFETY SESSIONS · LED BY VP QUALITY (NICK MATHERS)



## MANDATORY MONTHLY RCA — ONE RESIDENT ASSIGNED PER PRESENTATION MONTHLY — WITH DEDICATED FACULTY MENTOR



### CURRICULUM STRUCTURE

- Fully redesigned under Nick Mathers & Dr. Abedi — 100% faculty attendance documented
- 100% resident participation in all 6 annual sessions — no exclusions, attendance tracked via QR code
- Leadership: Nick Mathers, VP Quality, Risk Mgmt & Safety (&Dr.Abedi)

### MONITORING & ASSESSMENT

- 2 residents assigned per monthly RCA — 100% resident participation across all sessions
- Monthly resident surveys confirm engagement and safety culture improvement
- Resident attendance at hospital quality meetings — documented and ongoing

# Citation Response Dashboard — All 6 Citations

Methodist Healthcare IM Residency | Response filed February 2026 | Site visit: September 2026

1

## Inadequate Didactic Curriculum

ABIM Blueprint rebuild · 22-domain curriculum · +9 hrs/wk · MKSAP aligned · Schedules 6–12 mo in advance · QR tracking

✓ COMPLETED

2

## Inadequate Faculty Responsibilities

100% attendance · 0 unplanned cancellations · 6–12mo advance scheduling · Reminders system · Monthly surveys

✓ COMPLETED

3

## Inadequate Progressive Responsibility

17-page Autonomy Policy · Quarter-by-quarter caps · Consultant engagement reset · Milestone-based criteria · NI embedded

✓ COMPLETED

4

## Inadequate Feedback & Evaluations

Completion rate 50% → 90% post new PD · QR real-time eval → NI · Competency-based redesign · Faculty dev sessions · Milestone-anchored documentation

✓ COMPLETED

5

## Inadequate Transitions of Care

Night float system · I-PASS standardized · 7am/7pm structured sign-outs · Dr. Chavan nocturnal lead · PD audits

✓ COMPLETED

6

## Inadequate Patient Safety Culture

1 RCA/month · 100% resident participation · Revamped QI & Patient Safety Curriculum · 100% faculty attendance · Resident attendance at hospital quality meetings

✓ COMPLETED

# Resident Performance

- ▶ In-Training Exam (ITE) Results: 2024 → 2025 Trend
- ▶ 2nd Place — Texas ACP Doctor's Dilemma® — Proof the Curriculum Works
- ▶ Individual Resident Scores & PGY-Level Trajectories
- ▶ Returning Resident Longitudinal Growth
- ▶ Milestone Achievement — AY 2024–2025
- ▶ Work Hours Compliance — Zero Violations

## In-Training Exam — Program Trend: 2024 vs. 2025

2024 ABIM IM-ITE® (n=14) → 2025 ABIM IM-ITE® (n=25) | Program 430036 | Compared to national means by PGY level

### PGY-1 2024

n=12

**61.8%**

52nd %ile

▲ At national mean

### PGY-2 2024

n=2

**68.5%**

53rd %ile

▲ At national mean

### PGY-1 2025

n=14

**52.6%**

22nd %ile

⚠ New class below mean

### PGY-2 2025

n=9

**65.7%**

45th %ile

▲ At national mean

### PGY-3 2025

n=2

**69.0%**

47th %ile

▲ At national mean

#### PGY-1 CONTENT AREA TRENDS · 2025 vs. National

▲ Cardiovascular: Program 14% | Nat'l 12%

▲ Infectious Disease: Program 8% | Nat'l 7%

● Gastroenterology: 8% | 8%

● Endocrinology: 7% | 7%

● Heme/Onc: 9% | 9%

● Rheumatology: 5% | 5%

▼ Pulm/Critical Care: 7% | 9%

▼ Nephrology: 6% | 7%

▼ Neurology: 6% | 7%

▼ General IM: 11% | 13%

▲ Above national ● At national ▼ Below national

#### PRIORITY GAPS · REMEDIATION PLAN

##### Priority Deficits (2025 PGY-1):

▼ Neurology -1% · Pulm/CC -2% · Gen IM -2%

#### BOARD REVIEW INTERVENTIONS:

- +1 hr/month board review by Dr. Gidwani
- More Medical Jeopardy sessions (Gamification)
- Focused additional monthly noon-conference board review

#### Individualized Learning Plan (ILP) (If Resident ITE <60%):

- 50 MKSAP Q/month sent by RESIDENT to PA for review/audit

**MAY 2026 SUPPLEMENTAL ITE · Additional ITE for PGY-1 class (22nd %ile) — all PGY levels will take it**

Goal: Quantify improvement after curriculum restructuring · Review at June 2026 CCC



# ITE 2025 — Individual Resident Scores (De-identified)

2025 ABIM IM-ITE® | All residents by PGY level | 🚩 = below 25th percentile | Names removed for external sharing

| PGY-1 (n=14) |                        | AVG: 52.6%              | 22nd %ILE |
|--------------|------------------------|-------------------------|-----------|
| PGY1-R01     | <div><div></div></div> | 60% (51 <sup>th</sup> ) |           |
| PGY1-R02     | <div><div></div></div> | 60% (51 <sup>th</sup> ) |           |
| PGY1-R03     | <div><div></div></div> | 59% (45 <sup>th</sup> ) |           |
| PGY1-R04     | <div><div></div></div> | 57% (36 <sup>th</sup> ) |           |
| 🚩 PGY1-R05   | <div><div></div></div> | 54% (23 <sup>th</sup> ) |           |
| 🚩 PGY1-R06   | <div><div></div></div> | 54% (23 <sup>th</sup> ) |           |
| 🚩 PGY1-R07   | <div><div></div></div> | 53% (19 <sup>th</sup> ) |           |
| 🚩 PGY1-R08   | <div><div></div></div> | 52% (16 <sup>th</sup> ) |           |
| 🚩 PGY1-R09   | <div><div></div></div> | 51% (13 <sup>th</sup> ) |           |
| 🚩 PGY1-R10   | <div><div></div></div> | 51% (13 <sup>th</sup> ) |           |
| 🚩 PGY1-R11   | <div><div></div></div> | 50% (10 <sup>th</sup> ) |           |
| 🚩 PGY1-R12   | <div><div></div></div> | 46% (3 <sup>th</sup> )  |           |
| 🚩 PGY1-R13   | <div><div></div></div> | 45% (3 <sup>th</sup> )  |           |
| 🚩 PGY1-R14   | <div><div></div></div> | 44% (2 <sup>th</sup> )  |           |

National mean ~61%

| PGY-2 (n=9) |                        | AVG: 65.7%              | 45th %ILE |
|-------------|------------------------|-------------------------|-----------|
| PGY2-R01    | <div><div></div></div> | 73% (78 <sup>th</sup> ) |           |
| PGY2-R02    | <div><div></div></div> | 72% (75 <sup>th</sup> ) |           |
| PGY2-R03    | <div><div></div></div> | 69% (61 <sup>th</sup> ) |           |
| PGY2-R04    | <div><div></div></div> | 65% (42 <sup>th</sup> ) |           |
| PGY2-R05    | <div><div></div></div> | 64% (37 <sup>th</sup> ) |           |
| PGY2-R06    | <div><div></div></div> | 64% (37 <sup>th</sup> ) |           |
| PGY2-R07    | <div><div></div></div> | 64% (37 <sup>th</sup> ) |           |
| PGY2-R08    | <div><div></div></div> | 62% (28 <sup>th</sup> ) |           |
| 🚩 PGY2-R09  | <div><div></div></div> | 58% (14 <sup>th</sup> ) |           |

National mean ~66%

| PGY-3 (n=2) |                        | AVG: 69.0%              | 47th %ILE |
|-------------|------------------------|-------------------------|-----------|
| PGY3-R01    | <div><div></div></div> | 72% (63 <sup>th</sup> ) |           |
| PGY3-R02    | <div><div></div></div> | 66% (31 <sup>th</sup> ) |           |

National mean ~70%

PGY-1: 10 of 14 residents (71%) below 25th %ile at start of PGY-1 — primary focus • PGY-2 & PGY-3 tracking at national average

# ITE — Returning Resident Trajectory 2024 → 2025 (De-identified)

Residents present in both exam cycles | Tracking individual growth as PGY level advances | Names removed for external sharing

| Resident ID | 2024 PGY | 2024 Score | 2024 %ile        | 2025 PGY | 2025 Score | 2025 %ile        | Δ Score   | Trajectory Note            |
|-------------|----------|------------|------------------|----------|------------|------------------|-----------|----------------------------|
| PGY2-R01    | PGY-1    | 78%        | 98 <sup>th</sup> | PGY-2    | 73%        | 78 <sup>th</sup> | ▼ 5 pts   | PGY-2 above national mean  |
| PGY2-R02    | PGY-1    | 72%        | 91 <sup>th</sup> | PGY-2    | 72%        | 75 <sup>th</sup> | 0 pts     | Consistent top performer   |
| PGY2-R03    | PGY-1    | 65%        | 70 <sup>th</sup> | PGY-2    | 69%        | 61 <sup>th</sup> | ▲ +4 pts  | +4 pts                     |
| PGY2-R04    | PGY-1    | 66%        | 74 <sup>th</sup> | PGY-2    | 65%        | 42 <sup>th</sup> | ▼ 1 pts   | Stable; PGY-2 mean higher  |
| PGY2-R07    | PGY-1    | 64%        | 66 <sup>th</sup> | PGY-2    | 64%        | 37 <sup>th</sup> | 0 pts     | Stable score               |
| PGY2-R05    | PGY-1    | 57%        | 32 <sup>th</sup> | PGY-2    | 64%        | 37 <sup>th</sup> | ▲ +7 pts  | +7 pts — strong growth     |
| PGY2-R06    | PGY-1    | 58%        | 38 <sup>th</sup> | PGY-2    | 64%        | 37 <sup>th</sup> | ▲ +6 pts  | +6 pts — strong growth     |
| PGY2-R08    | PGY-1    | 52%        | 14 <sup>th</sup> | PGY-2    | 62%        | 28 <sup>th</sup> | ▲ +10 pts | +10 pts — excellent growth |
| PGY2-R09    | PGY-1    | 52%        | 14 <sup>th</sup> | PGY-2    | 58%        | 14 <sup>th</sup> | ▲ +6 pts  | +6 pts; focused support    |
| PGY3-R01    | PGY-2    | 72%        | 70 <sup>th</sup> | PGY-3    | 72%        | 63 <sup>th</sup> | 0 pts     | Consistent PGY-3           |
| PGY3-R02    | PGY-2    | 65%        | 36 <sup>th</sup> | PGY-3    | 66%        | 31 <sup>th</sup> | ▲ +1 pts  | +1 pt                      |

KEY FINDING: Of 9 PGY-2 residents who also took the 2024 exam as PGY-1s, 7 improved raw scores (avg +4.7 pts). Longitudinal trajectory positive.

ITE 2025 — PGY-2 Content Area Performance vs. National

*n=9 PGY-2 residents | Program average vs. 2025 national PGY-2 mean by content area | Mock ITE May & Aug 2026 pending*

| CONTENT AREA       | Difference from National Mean (percentage points) — PGY-2 2025 (n=9) | Δ pts |
|--------------------|----------------------------------------------------------------------|-------|
| Infectious Disease | <div></div>                                                          | ++7.1 |
| Cardiology         | <div></div>                                                          | ++1.2 |
| Endocrinology      | <div></div>                                                          | ++0.5 |
| Rheumatology       | <div></div>                                                          | 0.0   |
| Hematology/Onc     | <div></div>                                                          | -0.8  |
| Neurology          | <div></div>                                                          | -2.0  |
| HVC                | <div></div>                                                          | -2.8  |
| Pulm/CC            | <div></div>                                                          | -2.8  |
| Nephrology         | <div></div>                                                          | -2.9  |
| Gen. Int. Medicine | <div></div>                                                          | -4.7  |

STRENGTHS: Infectious Disease (+7.1 pts above national mean) | PRIORITY GAPS: Gen. IM -4.7 · Nephrology -2.9 · HVC -2.8 · Pulm/CC -2.8 · Neurology -2.0

# 2nd Place — State of Texas

ACP Doctor's Dilemma® · Medical Knowledge Jeopardy Competition · 2025

## THE STORY

Methodist Healthcare IM competed in the ACP Doctor's Dilemma® and finished 2nd in the State of Texas in 2025.

The team narrowly missed advancing to Nationals, losing in the final round.

### Why this matters

Citation #1 flagged an unstructured curriculum with no ABIM alignment. A 2nd-place state finish in the same academic year as the rebuild is direct evidence the new didactic infrastructure works.

— *Bravein Amalakuhan, MD | Program Director*

*"We rebuilt our curriculum aggressively — from structure to volume — with a relentless focus on education and resident growth. This is the program we are building."*

## COMPETITION DETAILS

|             |                                        |
|-------------|----------------------------------------|
| Competition | ACP Doctor's Dilemma®                  |
| Level       | State of Texas — Regional Competition  |
| Result      | 2nd Place — Narrowly missed Nationals  |
| Year        | 2025 · AY 2024–2025                    |
| Format      | Jeopardy-style · All IM subspecialties |

## DIRECT ACGME CITATION #1 EVIDENCE

### Before (September 2025)

Unstructured curriculum · minimal ABIM alignment

### Action (within 90 days)

13.5 hrs/wk structured · ABIM-mapped · 22 expert-led domains

### Result (2025 AY)

2nd Place — State of Texas. Board-level knowledge within 1 year.

### What ACGME will ask

"How do you know your curriculum is working?" — the result answers it.

## Milestones & CCC — Structured Bi-Annual Review

CCC convened June 2025 & December 2025 | June 2026 CCC pending | ACGME IM Milestones 2.0

### MILESTONE REVIEW — PROCESS

#### CCC Structure

- Bi-annual CCC: June 2025 & December 2025
- Quorum of core faculty at every meeting
- All milestones submitted to ADS within required window

#### Assessment Methodology

- ACGME IM Milestones 2.0 behavioral anchors (Levels 1–5)
- Faculty trained on Level 2/3/4 distinctions via dedicated modules

#### Documentation Standard

- Narrative templates with milestone-specific language
- Observable behaviors documented — not impressions
- ILP goals set at each review cycle

#### Expected Trajectory

- PGY-1: Level 1–2 | PGY-2: Level 2–3 | PGY-3: Level 3–4

### MILESTONE SUMMARY — AY 2024–2025 (CCC reviewed June 2025 & December 2025 · June 2026 CCC pending)

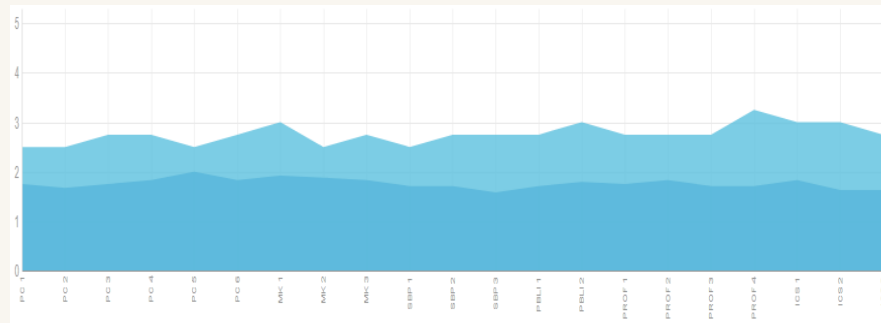
- CCC convened June 2025 and December 2025 with full quorum
- All milestone data submitted to ADS within required window
- Narrative evaluations use Level 2/3/4 behavioral language
- Every resident has an active ILP tied to milestone gaps
- Bi-annual cadence ensures continuous trajectory tracking

## Milestone Achievement — Aggregate Data for AY 2024-2025, CCC June 2025

14 Residents • PGY-1 & PGY-2 / PGY-1 & PGY-2 progression across all 21 ACGME IM Milestones 2.0 sub-competencies



Radar view — all sub-competencies



Stacked view — PGY-1 (dark) vs. PGY-2 (light)

### KEY TAKEAWAY

- **PGY-1 avg: Level 1.5–2** (on trajectory)
- **PGY-2 avg: Level 2.5–3** (meeting graduation-readiness pace)
- Uniform progression across all 6 competency domains
- No outliers — CCC calibration driving consistency

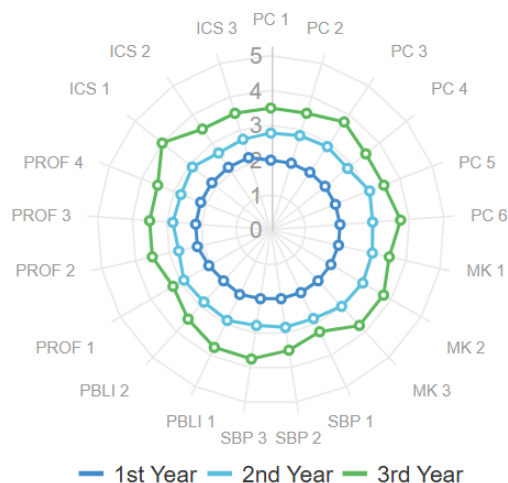
## Milestone Achievement — Aggregate Data for AY 2025-2026, CCC Dec 2025

25 Residents · PGY-1, PGY-2 & PGY-3 / Progression across all 21 ACGME IM Milestones 2.0 sub-competencies

December 2025

25 Residents

Program Director: Amalakuhan, Bravein



### KEY TAKEAWAY

- **PGY-3 avg** Level 3.5–4 across domains
- **PGY-2 avg** Level 3
- **PGY-1 avg** Level 2
- Clear stepwise progression across PGY levels
- July 2025 Cohort grew 14 → 25 residents (PGY-3 class added)
- No residents flagged for remediation

# Work Hours — AY 2024–2025, AY 2025-2026

ACGME duty hour compliance | Monitored weekly via New Innovations



NO DUTY HOUR VIOLATIONS REPORTED — AY 2024–2025, AY 2025-2026

PLATFORM

New Innovations — mandatory weekly duty hour logging for all residents

REVIEW FREQUENCY

Program Administrator reviews weekly · Program Director reviews monthly

ESCALATION PROTOCOL

Potential violation reviewed within 48 hours by Program Director · DIO notified per HCA policy if confirmed

RESIDENT EDUCATION

Duty hour policies reviewed at orientation · Reinforced at each semi-annual evaluation



# Research Volume — Resident Scholarly Activity

42 active projects as of April 2026 — an ~8× increase from June 2025 (5 projects) · +6 additional projects pending approval in addition currently

42+

Active Projects

14+

Residents Engaged

12+

Faculty Mentors

8×

Growth since 6/2025

## BY PROJECT TYPE

|                       |    |
|-----------------------|----|
| ■ Case Reports        | 20 |
| ■ Educational         | 13 |
| ■ Research            | 8  |
| ■ Quality Improvement | 1  |

## BY STATUS

|                           |    |
|---------------------------|----|
| ■ Conference Submissions  | 24 |
| ■ Presented               | 16 |
| ■ Published / In Analysis | 2  |

## BY PGY LEVEL

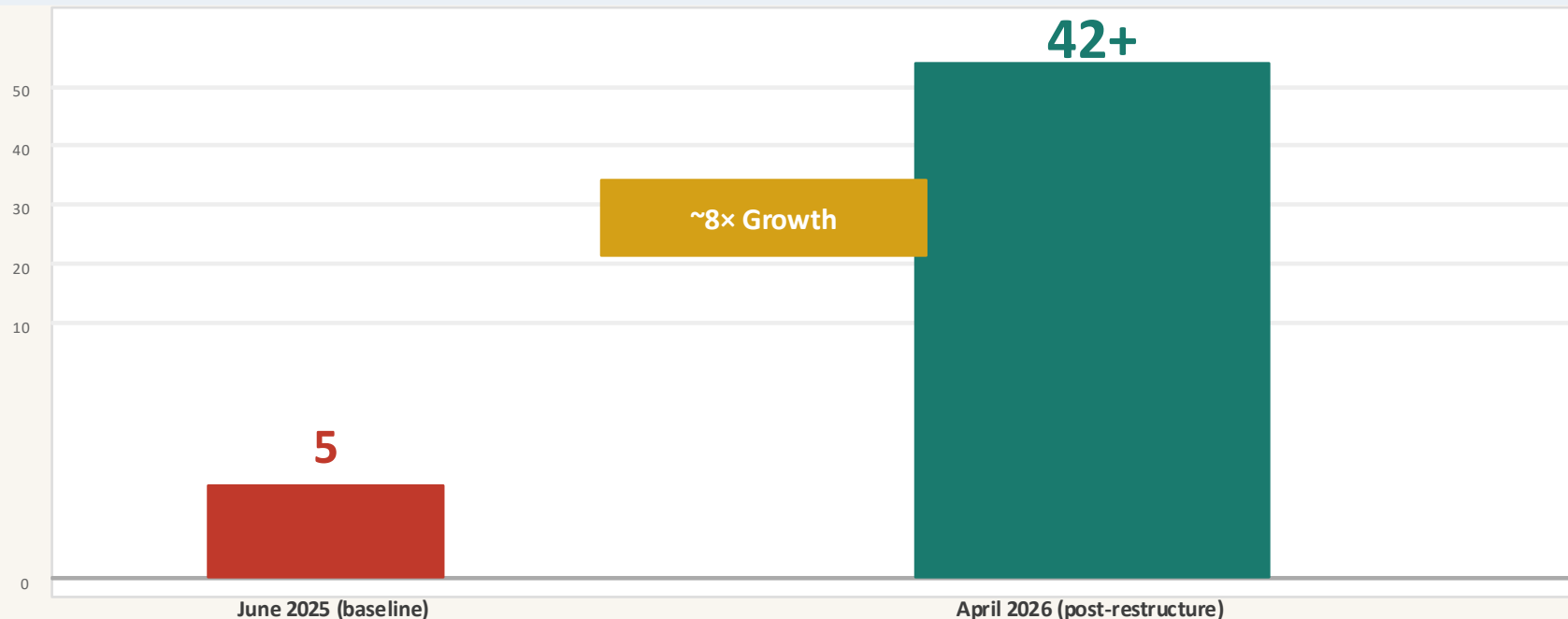
|         |    |
|---------|----|
| ■ PGY-1 | 8  |
| ■ PGY-2 | 29 |
| ■ PGY-3 | 5  |

PD mentoring 25 of 42 projects (60%)

6/2025: 5 active projects → 4/2026: 42 active projects (≈8× growth in < 1-year)

## Research Program Growth — Before vs. After PD Restructure

Active projects at June 2025 baseline: 5 | After PD restructure (April 2026): 42 projects | ~8× growth



**KEY DRIVERS:** PD-built research infrastructure · PD mentoring 25 of 42 projects (60%) · Formalized faculty scholarly expectations · Culture of academic achievement established

## MISSION MOMENT · THE EDUCATION SYSTEM WORKS

# Building Research Volume: From 5 to 42 Projects in < 1-year

**OBSERVATION:** Fewer than 5 active projects program-wide; less structured mentorship in AY 2024–2025.

**REVIEW:** Capability existed — infrastructure, mentorship, and culture did not.

**EDUCATION:** Formalized scholarly expectations & goals; built research curriculum and dedicated research rotation.

**CHANGE:** Created ‘Research Faculty Dept’ & appointed a ‘Senior Research Scholar’; resident-faculty pairings · project/conference registry · submission tracking.

**RESULT:** 42 active projects by April 2026 (~8× growth); 16 national abstract acceptances.

### THE RESULT

8×

Research growth in <1 year

5 → 42

active projects · June 2025 → Apr 2026

16

national abstracts accepted

ACC · SCAI · DDW · AAN · SCCM · ASCO · ACP

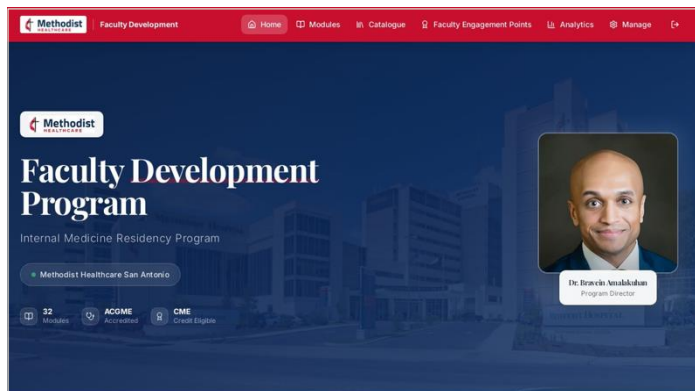
# Program Quality& Innovation

- ▶ Resident Wellness Events — 5 Structured Group Events AY 2025–2026
- ▶ ACGME Survey Preparation & Expected Outcomes
- ▶ Recruitment: 15/15 Spots Filled — 325 Interviewed, 302 Ranked
- ▶ Health Equity & Inclusion — San Antonio's Majority-Minority Population
- ▶ Core Knowledge Curriculum: 22 ACGME Domains, ABIM Blueprint-Aligned
- ▶ Infrastructure: 6,400 sq ft Education Center Opening July 2026
- ▶ Didactics & Clinical Curriculum: ~720 Structured Hours/Year

# Resident Wellness Events — AY 2025–2026

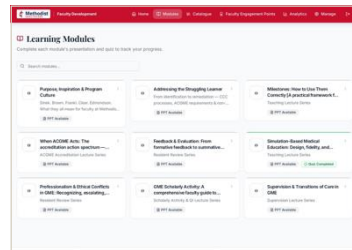
*Structured group events build culture, cohesion, and identity · Wellness is a program commitment, not an afterthought*

- |    |                                                                                                     |
|----|-----------------------------------------------------------------------------------------------------|
| 01 | May 3, 2025 <b>Murder Mystery Dinner</b> · Team Building & Social Event                             |
| 02 | Jun 20, 2025 <b>Andretti Go Kart &amp; Arcade</b> · Group Racing & Arcade Night                     |
| 03 | Aug 29, 2025 <b>Chicken N Pickle</b> · Pickleball Night — Group Social                              |
| 04 | Oct 28, 2025 <b>National IM Day</b> · Gift for residents celebrating IM                             |
| 05 | Oct 31, 2025 <b>Six Flags Fright Night</b> · Halloween Group Outing                                 |
| 06 | Dec 18, 2025 <b>Christmas Party</b> · Resident Holiday Celebration & Dinner                         |
| 07 | Mar 5, 2026 <b>Medical Jeopardy</b> · Games, Lunch & Team Building                                  |
| 08 | Mar 23, 2026 <b>Lil Woodrow's Social &amp; Appreciation</b> · Games, Activities & Team Appreciation |
| 09 | Apr 9, 2026 <b>Fogo de Chão — Brazilian Steakhouse</b> · Program Celebration Dinner                 |
| 10 | May 29, 2026 <b>Pottery &amp; Painting Night</b> · Creative Arts Team Bonding                       |



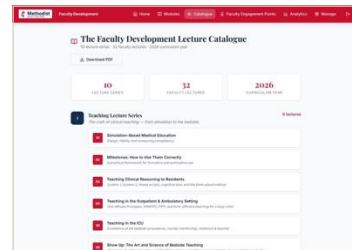
## 01 Homepage & Program Identity

Faculty Development Lectures: 32 modules, ACGME-accredited, CME-eligible.



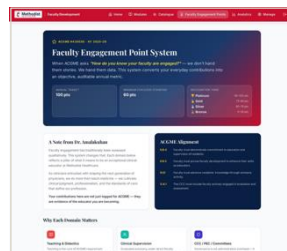
## 02 Modules Library

32 branded learning modules with integrated quizzes and progress tracking.



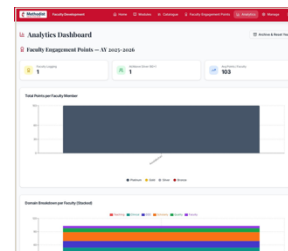
## 03 Lecture Catalogue

10 lecture series, faculty development curriculum, 2026 academic year.



## 04 Engagement Points

ACGME-aligned faculty engagement metric — objective, auditable, tiered (Platinum / Gold / Silver / Bronze). Log Page



## 05 Analytics Dashboard

Real-time faculty engagement analytics — domain breakdown, tier distribution, trends.

## Wellness — AY 2024–2025

### RESIDENT WELLNESS

- 4 structured wellness activities per AY (built into calendar)
- Hiking · Lunch/Dinners off campus · Six Flags · Socials
- Holiday & cultural celebrations reflecting resident diversity
- Psychological safety — raise concerns without reprisal
- Semi-annual PD evaluations include wellness check-in
- Chief residents as first-line wellness resource
- Duty hour compliance maintained — no violations
- PD open-door policy · Chaplaincy · Peer support network

### FACULTY WELLNESS

- Faculty wellness = program priority (burnout impairs teaching)
- Curriculum on purpose & meaning (Frankl / Sinek / Brown / Clear)
- Direct PD access — faculty concerns raised openly
- Faculty meetings build collegiality alongside agenda
- Equitable teaching & evaluation workload — actively monitored
- HCA EAP: free counseling (8 sessions/yr), financial, legal — 24/7

## GME Continuity Clinic: #1 Quality Performer at Methodist Health

**OBSERVATION:** Inconsistent chronic disease management and quality documentation gaps across Methodist clinics.

**REVIEW:** HEDIS and Methodist dashboards showed GME clinic below system average at baseline.

**EDUCATION:** Didactic & QI curriculum redesigned with Nick Mathers (VPQ) and Dr. Ali Abedi (GME Dir. of Patient Safety) — monthly RCAs, PDSA cycles, Z-codes, NASEM 5As SDOH framework.

**CHANGE:** Layered education and mentorship by Dr. Paz (Director of Outpatient Medicine) shifted the clinic culture; residents closed care gaps, drove protocol adherence, and embedded SDOH screening into every visit.

**RESULT:** Curriculum redesign translated directly into measurable outcomes — our didactic & educational changes in motion, proven by dashboard data.

### THE RESULT

# #1

Quality-Performing Clinic at Methodist  
Healthcare

Driven by GME residents & faculty

**GAPS → EDUCATION → OUTCOMES**

*ACGME PBLI & SBP milestones in action*



# From Didactic Rebuild to Clinical Excellence

When we rebuilt the curriculum, wrote the QI handbook, and taught quality measures at the bedside — our residents brought it back to the clinic. The data tells the story.

"Quality performance is not a byproduct of good residents — it is evidence that the program is teaching them to practice medicine the right way."

|                                                                                                               |                                                                                                           |                                                                                                                                |                                                                                                                             |
|---------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------|
| <p><b>#1</b></p> <p>Diabetes Control<br/>in San Antonio Market</p> <p>QM 1 · HbA1C &gt; 9%<br/>March 2026</p> | <p><b>100%</b></p> <p>MIPS Compliance<br/>Merit-Based Incentive Payment</p> <p>Goal: 95%<br/>YTD 2026</p> | <p><b>100%</b></p> <p>PDMP Compliance<br/>Prescription Drug Monitoring</p> <p>Every resident.<br/>Every Rx. No exceptions.</p> | <p><b>83.1</b></p> <p>Overall MIPS Score<br/>QPI KRA Dashboard</p> <p>Enterprise avg: 82.1<br/>Quality: 28.1 (avg 26.5)</p> |
|---------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------|

THE TRANSFORMATION · WHAT RESIDENTS + FACULTY ACCOMPLISHED TOGETHER · AY 2024–2026

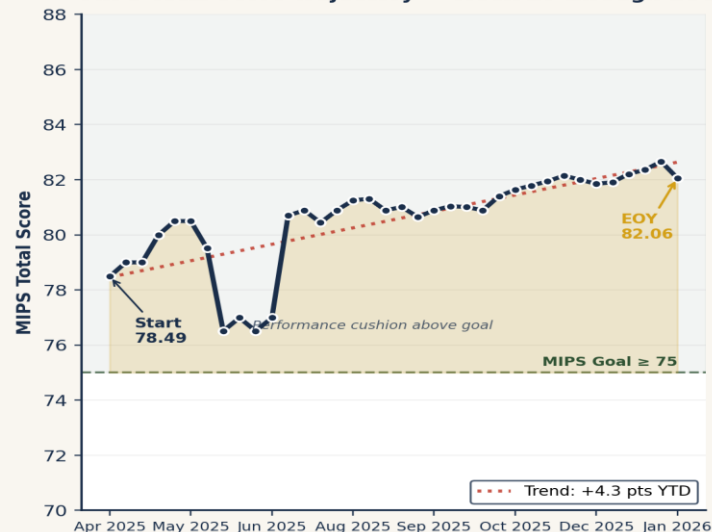
|        |                                                                         |                 |                       |                    |            |
|--------|-------------------------------------------------------------------------|-----------------|-----------------------|--------------------|------------|
| QM 134 | Depression Screening<br>14 months · Dec 2024 → Mar 2026 ·               | Before<br>0%    | After (2026)<br>91.6% | Goal<br>> 65%      | ✓ EXCEEDED |
| QM 1   | Diabetes HbA1C Control<br>INVERSE metric · LOWER is BETTER · #1 in City | Before<br>44.7% | After (2026)<br>12.2% | Goal<br>< 28.5%    | ✓ EXCEEDED |
| QM 236 | Hypertension Control<br>Trending up month over month                    | Before<br>65%   | After (2026)<br>73.1% | Goal<br>> 75%      | ✓ EXCEEDED |
| QM 318 | Falls Risk Screening (≥65)<br>Sustained above benchmark Jan–Mar 2026    | Before<br>77%   | After (2026)<br>91%   | Goal<br>HEDIS std. | ✓ EXCEEDED |

GME Continuity Clinic (MPP Practice 22497) — #1 Quality-Performing Clinic in the Methodist Healthcare System · Source: QPI KRA Dashboard YTD 2026

# GME Clinic: A Year of Measurable Quality Gains

*Dr. Paz & Residents drove continuous improvement across 37 weeks of MIPS reporting — finishing 2025 above every benchmark.*

**MIPS Total Score Trajectory — Week 16 through EOY 2025**



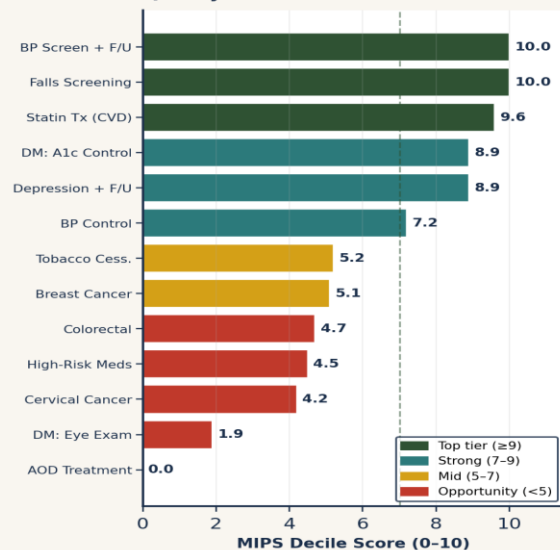
**+3.57**

MIPS Total Points Gained  
(Week 16 → EOY)

**+3.57**

Quality Score Gain  
(Goal > 25 — surpassed)

**Quality Measure Portfolio — EOY 2025**



**82.21**

CLINIC MIPS TOTAL SCORE • Goal ≥ 75 • +7.21 above benchmark

**100%**

PROVIDER MIPS COMPLIANCE • 1 of 1 providers met • Goal ≥ 90.5%

**2 × 10.0**

PERFECT-DECILE MEASURES • BP Screening & Falls Screening

★ **Decile 10** = top 10% of clinicians nationally on that CMS measure

# How a Didactic Rebuild Became a #1 Clinic

*The through-line from classroom to chart: every clinical quality gain is traceable to a specific educational intervention delivered by the program.*

## 01

### The Curriculum Rebuild

22-domain ABIM-aligned didactic program. 13.5 hrs/wk — +238% from baseline. MKSAP 2025–26 mapped. Zero unplanned cancellations.

## 02

### QI & Patient Safety Teaching

Nick Mather's redesigned QI curriculum: monthly RCA sessions, PDSA cycles, Z-code documentation, NASEM 5As SDOH framework, bedside coaching.

## 03

### Clinical Reasoning at the Bedside

Faculty coached residents on chronic disease protocols, HEDIS/MIPS measures, and quality documentation during every clinic session.

## 04

### Resident-Led Execution

Residents embedded PHQ-9, HbA1C tracking, BP management, falls screening, and SDOH screening into every outpatient encounter.

## 05

### Measurable Clinical Outcomes

Depression screening 0 → 91.6%. Diabetes #1 in SA market. MIPS 83.1. PDMP 100%. Quality score 28.1 (enterprise avg 26.5).

## ACGME COMPETENCY DOMAINS DEMONSTRATED · QUALITY AS A PROGRAM OUTCOME

|            |                                                                                                                                                                                                                                 |
|------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| PC · MK    | <b>Patient Care · Medical Knowledge</b><br>Depression screening 0→91.6%. Diabetes control #1 in San Antonio market. Hypertension control 73.1%, approaching 75% goal.                                                           |
| PBL · SBP  | <b>Practice-Based Learning · Systems-Based Practice</b><br>MIPS score 83.1 (above enterprise avg). 100% PDMP compliance. Quality score 28.1 (avg 26.5). Program-wide culture shift reflected in YTD green across every measure. |
| IPS · PROF | <b>Interpersonal Skills · Professionalism</b><br>Honest gaps acknowledged: breast & colorectal cancer screening below goal — active QI targets for 2025–26. Demonstrates program maturity and ACGME self-assessment competency. |

*The didactic rebuild did not end at the noon-conference door. It traveled with every resident into every exam room — and became measurable clinical care.*

## ACGME Survey Results — Program Preparation Summary

*Survey period: February 9 — April 10, 2026 | Results Pending*

ACGME ANNUAL SURVEY — Survey period: Feb 9 — Apr 10, 2026 · Results available from ADS portal post-April 10, 2026 · To be inserted upon release. Expected domains: Program Director effectiveness · Faculty teaching quality · Curriculum comprehensiveness · Feedback quality · Wellness · Supervision · Patient Safety culture · Overall program satisfaction

### WHAT THE DATA WILL SHOW — EXPECTED OUTCOMES

#### Resident Survey Readiness

Residents briefed · completion deadline April 10, 2026 · reminder emails sent

#### Faculty Survey Readiness

Faculty briefed · All 14 Faculty Development modules delivered · Survey responses reflect program improvements

#### Expected Improvement Areas

Faculty teaching · Milestones · Feedback · Curriculum — all addressed by AY 25-26 changes

# Matched Applicants — Class of 2029 (Starting July 2026)

NRMP Match Day, March 2026 | 15/15 spots filled | 325 interviewed, 302 ranked | Source: NRMP Match results

**MD/DO:** 9 MD / 6 DO (60/40%)

**Avg Step 2:** 234

**Avg COMLEX 2:** 473

**Signals:** 2 Gold · 5 Silver

## MATCHED RESIDENT | Medical School | Degree

- **Neha Gurram, MD** American Univ. of Antigua
- **Laxmi Mahita Paripati, MD** Malla Reddy Inst. of Med. Sciences
- **Chang-Rong Chen, MD** Univ. Vita-Salute San Raffaele (Italy)
- **Yara Alhaddadein, MD** Mu'tah University (Jordan)
- **Michelle Rakaba, DO** UIW School of Osteopathic Medicine
- **Mitchell DeVolder, MD** Texas Tech Univ. Health Sciences Center
- **Travis Quillin, DO** Burrell College of Osteopathic Medicine
- **Stephen Yekini, MD** Univ. of Southampton (UK)

## MATCHED RESIDENT | Medical School | Degree

- **Rachel Joseph, DO** Kansas City Univ. College of Osteopathic Med.
- **Precious Omokaro, DO** Edward Via College of Osteopathic Med. — LA
- **Matthew King, MD** Texas A&M Univ. Naresh K Vashisht SOM
- **Sneha Suresh, DO** Arkansas College of Osteopathic Medicine
- **Yazan Hamadneh, MD** Univ. of Jordan Faculty of Medicine
- **Lipilekha Mukherjee, MD** Univ. of Kansas School of Medicine — Wichita
- **Sasanka Thupili, DO** Kansas City Univ. College of Osteopathic Med.

## Resident Recruitment — AY 2025–2026 Season (Class Starting July 2026)

### Candidates Interviewed

325

#### Spots Filled — Initial MATCH

15

#### Spots Filled via SOAP

0

#### Total Filled

15

#### MD / DO Split

60% MD / 40% DO

#### Avg USMLE Step 2 / COMLEX 2 Score

Avg Step 2: 234 | Avg  
COMLEX 2: 473

#### Interview Exception Requests Submitted to DIO

None

### RECRUITMENT INITIATIVES

- Rebuilt recruitment presentation — updated with new program vision, residents, and city highlights
- Instagram account launched for resident-driven content and recruitment pipeline
- 3 open houses conducted (virtual capability available)
- Mentorship program: applicants connected with current residents
- Gender, photo, and ethnicity screening/blinding during application review
- Active recruitment of diverse candidates mirroring San Antonio demographics

### 2025–2026 RECRUITMENT PLAN

Interviews: Fall 2025 | Format: Virtual + in-person GME space option | Open houses: x3 completed (11/4/25 · 12/3/25 · 1/16/26) | Applications received: 1,440 | Met filters: ~600 | Interview ratio: 25:1 (325 total interviews / 15 slots) | Mentorship: Continued and expanded | Application review: Electronic with blinding protocol

## Health Equity & Inclusion — AY 2025–2026

### IN RESIDENT RECRUITMENT

- Blinded application review — reduces implicit bias
- Intentional recruitment of residents native to San Antonio or with family ties
- Mentorship for all applicants — levels the field
- Accessible, welcoming open houses
- Mission: serve the underserved in San Antonio

### IN FACULTY

- Leaders recruit faculty through patient lens
- Faculty diversity = clinical & educational asset
- Diverse backgrounds, genders, cultural perspectives
- Cultural competency & health equity focus

### IN THE CLINICAL LEARNING ENVIRONMENT

- Private space for worship, prayer, meditation
- SDOH-integrated curriculum addresses inequities
- 3 SDOH faculty modules — system leader in equity
- Psychological safety — bring authentic selves
- Cultural celebrations throughout the year
- Spanish-speaking residents = clinical asset

# Core Knowledge Curriculum — ACGME CPR IV.A.3.c Aligned

22 ACGME core domains | Rebuilt post-site visit | Expert faculty-led | Tracked via Curriculum Tracker | PEC reviewed every 6 months

| DOMAIN                         | TEACHING MODALITY                                                                       | FREQUENCY / OWNER                      |
|--------------------------------|-----------------------------------------------------------------------------------------|----------------------------------------|
| General Internal Medicine      | Morning Report, Noon Conf, Inpatient Rounds, Board Review, Thursday Didactics, Clinic   | Daily, Weekly / Chiefs & Faculty       |
| Cardiology                     | Cardiology Conf, Noon Conf, Simulation, Morning Report, Board Review                    | Daily, Weekly / Cardiology Faculty     |
| Pulmonary & Critical Care      | ICU Didactics, Simulation, Procedure Lab, Morning Report, Noon Conf, Thursday Didactics | Daily, Weekly / Chiefs & Faculty       |
| Gastroenterology & Hepatology  | Noon Conf, Case-Based, Morning Report, Thursday Didactics, Board Review                 | Daily, Weekly, Monthly / GI Faculty    |
| Nephrology                     | Noon Conf, Case-Based, Morning Report, Thursday Didactics, Board Review                 | Daily, Weekly, Monthly / Renal Faculty |
| Endocrinology                  | Morning Report, Noon Conf, Inpatient Rounds, Board Review, Thursday Didactics, Clinic   | Daily, Weekly / Chiefs & Faculty       |
| Infectious Disease             | Noon Conf, Case-Based, Morning Report, Thursday Didactics, Board Review                 | Daily, Weekly / ID & General Faculty   |
| Rheumatology & Immunology      | Noon Conf, Case-Based, Morning Report, Thursday Didactics, Board Review                 | Daily, Weekly / Faculty                |
| Hematology / Oncology          | Noon Conf, Case-Based, Morning Report, Thursday Didactics, Board Review, Tumor Board    | Daily, Weekly / Heme-Onc & Faculty     |
| Neurology                      | Noon Conf, Case-Based, Morning Report, Thursday Didactics, Board Review                 | Daily, Weekly / Faculty                |
| Geriatrics & Palliative Med    | Noon Conf, Case-Based, Morning Report, Thursday Didactics, Clinic                       | Daily, Weekly / Faculty                |
| QI & Patient Safety            | Noon Conf, Case-Based, Morning Report, Thursday Didactics, M&M                          | Monthly, Quarterly / QI Director       |
| Health Systems Science         | QI Didactics, Interdisciplinary Rounds                                                  | Quarterly / Core Faculty               |
| EBM & Research                 | Journal Club, Research Didactics, Research Course                                       | Monthly / Research Faculty             |
| Ethics, Prof, Health Equity    | Bioethics Grand Rounds, Thursday Didactics                                              | Weekly, Monthly, Quarterly / Faculty   |
| Allergy & Immunology           | Noon Conf, Case-Based, Morning Report, Thursday Didactics, Board Review                 | Daily, Weekly / Faculty                |
| Dermatology                    | Interactive Lectures, Clinical-Path Correlation, Clinic Rotations                       | Quarterly / Faculty                    |
| Psychiatry / Behavioral Med    | Thursday Didactics, Morning Report, Clinic Didactics                                    | Daily, Quarterly / Faculty             |
| Preventive & Lifestyle Med     | Continuity Clinic, Clinic Didactics, Wellness Series, Thursday Didactics                | Weekly, Quarterly / Core Faculty       |
| Emergency Med (IM Perspective) | Simulation Lab, ACLS Workshops, Thursday Didactics, Morning Report, Noon Conf           | Quarterly / ICU & EM Faculty           |
| Musculoskeletal Medicine       | Clinic Didactics, Noon Conf, Morning Report, Thursday Didactics                         | Quarterly / Faculty                    |
| Ophthalmology / ENT            | Thursday Didactics, Morning Report, Noon Conf                                           | Quarterly / Faculty                    |
| Obstetric / Gyn Medicine       | Women's Health Didactics, Outpatient Clinic                                             | Quarterly / OB-GYN & IM Faculty        |

Curriculum Tracker monitors annual completion of all 22 ACGME core domains · PEC reviews didactic schedule every 6 months · Monthly/quarterly resident feedback sessions conducted · Meets CPR IV.A.3.c



# Noon Conference Didactic Module Index — AY 2025–2026

Expert faculty-led · MKSAP 2025–2026 aligned · Mirrors ACP Board Review · ABIM blueprint-mapped

## CARDIOLOGY ×5

**Cardiology 1** Chest Pain, ACS, Stable Angina  
**Cardiology 2** HFrEF, HFpEF, Acute Decomp HF  
**Cardiology 3** Valvular Disease, Prosthetics  
**Cardiology 4** Pericardial, Endocarditis, Aortic, PAD  
**Cardiology 5** Bradycardia, Tachyarrhythmias

## ENDOCRINOLOGY ×7

**Endo 1–2** Thyroid  
 (Hypo/Hyper/Storm/Cancer)  
**Endo 3–4** Adrenal, Cushing, Pituitary, Hypogonadism  
**Endo 5** Calcium, Osteoporosis, Vitamin D  
**Endo 6–7** Diabetes  
 (T1/T2/DKA/HHS/Neuropathy)

## GASTROENTEROLOGY ×7

**GI 1–2** Esophagus, GERD, Barrett, PUD, H. pylori  
**GI 3–4** Biliary, Pancreatitis, Cholangitis  
**GI 5** Hepatitis, Cirrhosis, SBP, MASH, Liver Failure  
**GI 6–7** GI Bleed, IBD, Diarrhea, Malabsorption

## HEMATOLOGY ×5

**Hem 1–2** Anemias, Sickle Cell, Transfusion Medicine  
**Hem 3–4** Thrombocytopenia, ITP, TTP, HIT, VTE  
**Hem 5** CML, MDS, AML, ALL, Myeloproliferative

## INFECTIOUS DISEASE ×7

**ID 1–2** SSTI, Osteomyelitis, Viral/Bacterial  
**ID 3–4** Pneumonia, UTI, CNS Infections, C. diff  
**ID 5** HIV, Opportunistic Infections  
**ID 6–7** HAI, TB, Fungal, Tick-Borne, Malaria

## NEPHROLOGY ×5

**Neph 1** HTN, Hypertensive Emergency, Renovascular  
**Neph 2–3** AKI, CKD, Nephrotic/Nephritic, Rhabdo  
**Neph 4–5** Electrolytes, Acid-Base Disorders

## PULMONARY / CC ×7

**Pulm 1–2** Asthma, COPD, Bronchiectasis  
**Pulm 3–4** ILD, Sarcoid, Pleural, OSA, Nodule  
**Pulm 5–6** PH, Sepsis, Anaphylaxis, Angio  
**Pulm 7** NIV, Invasive Vent, ARDS

## RHEUM ×5 · ONC ×4 · NEURO ×4

**Rheum 1–5** OA, RA, Gout, SLE, Vasculitis, MSK Pain  
**Onc 1–2** Lung, Breast, Colorectal, Lymphoma, Prostate  
**Onc 3–4** Febrile Neutropenia, Tumor Lysis, Palliative  
**Neuro 1–4** Headache, Stroke, Seizure, Parkinson, Dementia

# Institutional Infrastructure Investment — AY 2024–2025

Physical and operational investments supporting GME growth, resident experience, and ACGME accreditation readiness

## NEW EDUCATION CENTER

- 6,400 sq ft multipurpose facility
- 232-seat auditorium
- Conference rooms + classrooms
- Opens July 1, 2026 (SL1)
- Purpose-built academic space dedicated to GME education — signals long-term institutional commitment

## RESIDENT WORKROOMS

- MH 7th Floor Workroom — updated
- Metro 3rd Floor Workroom — new
- Sleep rooms: SL1, 15 rooms
- GME Suite: 7,900 sq ft (1st Floor Methodist Plaza)
- Dedicated resident spaces across hospital network — ACGME-compliant learning environment

## FELLOWSHIP PIPELINE

- Cardiology Fellowship — target 2027
- PCCM Fellowship — target 2028
- GI Fellowship — target 2028
- Prerequisite: Continued Accreditation status (Sept 2026 site visit). Program growth directly enables fellowship development.

## NIGHT FLOAT & CLINICAL OPERATIONS

- Night Float (Started 4/2025): Mon–Fri 7pm–7am; residents attends Morning Report
- ER–Inpatient admission process standardized (9/22/25)
- Continuity Clinic: NIMA Clinic partnership operational, begins 7/1/2026 [2 Continuity Clinics: Plaza + NIMA]

## NEW ROTATION SITES (AY 2024–2025)

- Methodist Hospital Landmark · Westover Hills · Metropolitan · Texusan/MSTH
- Elective expansion: CHF, Geriatrics, Pulmonary Transplant, Psychiatry, Endocrinology, LTAC, Sports Medicine
- Network now spans 6 acute care hospitals across MHS San Antonio

# Didactics & Clinical Curriculum Highlights — AY 2024–2025 (~720 Structured Hours/Year)

ABIM Blueprint-aligned · Mirrors ACP Board Review Programs · 13.5 hrs/week · ~720 hrs/year · All faculty-led

## ★ Board Review Curriculum

- Monthly expert board review (Dr. Gidwani: USMLE 280)
- Thursday Didactics Board Review Sessions
- MKSAP 2025–26 indexed noon conference
- Medical Jeopardy sessions (gamification)
- Internal Performance Plan: 50 MKSAP Q/mo if <60%
- May 2026 supplemental ITE — all PGY levels

## ★ Morning Report — Launched

- Start July 2025
- Daily structured case-based forum
- Residents present overnight cases
- Clinical reasoning & EBM focus

## ★ EKG Curriculum — Designed & Implemented

- Progressive EKG curriculum
- Fundamentals → advanced arrhythmias
- Mapped to ABIM blueprint

## ★ ACLS/BLS Simulation Course Series

- 4-part didactic + simulation
- Progressive resuscitation training
- Integrated into clinical schedule

## ★ Point-of-Care Ultrasound (POCUS) Course Series

- 8-part POCUS curriculum
- Simulation + didactic + hands-on
- Cardiac · lung · abdominal · vascular
- Aligned to PC-2 / PC-3

## ★ Noon Conference — Rebuilt + Expert Board Review

- Start December 2025
- ACGME-aligned Core Knowledge Curriculum
- 22 subspecialty domains
- MKSAP 2025–26 indexed
- Monthly expert board review
- Quarterly feedback to PEC

## ★ Four Specialty Handbooks — Created

- CVICU · ICU Handbooks
- Inpatient & Outpatient Medicine
- ABIM-grounded learning objectives
- Resident-level reference
- Will be loaded within a Digital Platform / App

## ★ SDOH Integration — New

- SDOH embedded across teaching
- Z-code documentation
- NASEM 5As framework
- 3 faculty modules
- 3 Resident modules

## Procedural Simulation + Emotional Intelligence Series

- Procedural simulation + EI/Leadership series
- ACLS simulation
- Patient Care simulation

## SECTION 04 · FACULTY PERFORMANCE

# Faculty Profiles & Highlights

- ▶ Program Director — Dr. Bravein Amalakuhan, MD, FCCP
- ▶ APD Director of Education — Dr. Jairo Melo, MD, FCCP (25+ years Methodist)
- ▶ APD Director of Strategic Growth — Dr. Fernando Triana, MD, FACC, FASPC
- ▶ APD Director of Clinical Operations — Dr. Shankar Sanka, MD
- ▶ Senior Research Scholar — Dr. Jeffrey DellaVolpe, MD, MPH (\$6.1M Active Grants)
- ▶ Director of Pulmonology & Patient Safety — Dr. Ali Abedi, MD, FCCP, DAABIP
- ▶ 2026 Methodist Healthcare GME Faculty Excellence Awards — 10 Recipients

# Bravein Amalakuhan, MD — Credentials & Leadership

ABIM Triple Board Certified · Triple Fellowship Certified · Pulmonary & Critical Care · Johns Hopkins + UT Health San Antonio

Program Director, Internal Medicine Residency

Medical Director, SICU/MICU & Pulmonary Services · Methodist Healthcare

✓ ABIM  
Critical Care Medicine  
2017

✓ ABIM  
Pulmonary Medicine  
2016

✓ ABIM  
Internal Medicine  
2014

| TRAINING PEDIGREE                                                                            |           |
|----------------------------------------------------------------------------------------------|-----------|
| Pulmonary & Critical Care Fellowship<br><i>UT Health San Antonio</i>                         | 2014–2017 |
| Cardiovascular Surgical Critical Care Fellowship<br><i>Johns Hopkins University Hospital</i> | 2013–2014 |
| Internal Medicine Residency<br><i>UPMC Harrisburg</i>                                        | 2010–2013 |
| Doctor of Medicine — Dean's List<br><i>St. George's University School of Medicine</i>        | 2006–2010 |

SCHOLARLY OUTPUT AT A GLANCE

15  
Accepted Abstracts  
AY 2025–2026

10  
Peer-Reviewed  
Publications

4  
Textbook  
Chapters

4  
Research Grants  
& Multi-Ctr Trials

| HONORS & AWARDS · 12 Recognitions · 2011–2026 |                                                                                                        |
|-----------------------------------------------|--------------------------------------------------------------------------------------------------------|
| 2026                                          | Clinical Leadership Excellence + Cardiovascular Innovation Award — Methodist Healthcare                |
| 2025                                          | Physician of the Quarter — Methodist Hospital Metropolitan                                             |
| 2021                                          | Affiliate Faculty Teacher of the Year — BAMC ER Residency                                              |
| 2018                                          | Rising Star Doctor Award — San Antonio Scene Magazine                                                  |
| 2016                                          | Top Oral Presentation — UT Health SA Medicine Research Day                                             |
| 2015                                          | Top Case Presentation (CHEST) · ATS Fellows Track Award (\$1,000) · Dean James J. Young Research Award |
| 2011–13                                       | Excellence in Research Award (×2) + 1st Place Oral & Poster — PinnacleHealth                           |

| HOSPITAL & NATIONAL LEADERSHIP                                |           |
|---------------------------------------------------------------|-----------|
| Chair, Resuscitation Committee<br><i>MH Metropolitan</i>      | 2025–     |
| Chief of Staff (2-yr term)<br><i>MH Texsan</i>                | 2023–2025 |
| Chair, Critical Care Committee<br><i>×2 Hospitals</i>         | 2019–     |
| Chair, Quality Excellence Committee<br><i>MH Metropolitan</i> | 2025–     |
| Medical Director, SICU/MICU<br><i>MH Metropolitan</i>         | 2022–     |
| CHEST Foundation Board of Trustees<br><i>National</i>         | 2021–2024 |

# Program Director Vision— To Build Scholars

Primary mentor on 28 of 45 active resident research projects · 16 confirmed accepted abstract publications co-authored with residents in AY 2025–2026 alone

## 28

of 45  
Resident Projects  
Mentored

## 16

Accepted Abstracts  
Co-Authored  
With Residents (AY25–26)

## 8

National Conferences  
Represented by  
Resident Co-Authors

## 100%

Abstract Acceptance  
Rate on Submitted  
Resident Manuscripts

AY 2025–2026 ACCEPTED ABSTRACTS — CO-AUTHORED WITH RESIDENTS · Amalakuhan B confirmed on all 16

|                                                                                                                                                                                                             |                                                                                                                                                                  |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>ACC 2026</b><br><b>×3</b><br><b>Peripartum Cardiomyopathy · HFpEF Microvascular Dysfunction · Hypertensive Heart/Kidney Disease</b><br><i>J Am Coll Cardiol. 2026;87(Suppl)</i>                          | <b>SCAI 2026</b><br><b>×2</b><br><b>WPW Syndrome Mortality Trends · Subclinical Atherosclerosis Imaging</b><br><i>J Soc Cardiovasc Angiogr Interv. 2026</i>      |
| <b>DDW 2026</b><br><b>×2</b><br><b>NAFLD/Obesity Mortality Trends · HCC + Ischemic Heart Disease Mortality</b><br><i>Gastroenterology. 2026;XXX(Suppl)</i>                                                  | <b>ASCO 2026</b><br><b>×2</b><br><b>Gallbladder Cancer Mortality · Pancreatic Cancer + Diabetes Mortality</b><br><i>ASCO Annual Meeting 2026</i>                 |
| <b>CHEST 2026</b><br><b>×3</b><br><b>ARDS-Sepsis Mortality (56% decline, 24yr) · COVID-19 Impact on Asthma Mortality · Cryptococcal Empyema (Amalakuhan 1st author)</b><br><i>CHEST Annual Meeting 2026</i> | <b>AAN 2026</b><br><b>×1</b><br><b>Seizure Recurrence After First Unprovoked Seizure</b><br><i>Neurology. 2026;XXX(Suppl)</i>                                    |
| <b>SCCM 2026</b><br><b>×1</b><br><b>Flexible Bronchoscopic Retrieval of Dental Drill Bit (with Chief Melo S)</b><br><i>Crit Care Med. 2026</i>                                                              | <b>Multi 2026</b><br><b>×2</b><br><b>COPD + T2DM Mortality (1999–2023) · Pulmonary HTN Mortality Pre/Post COVID</b><br><i>Multi-journal pending · CDC WONDER</i> |
| <b>Case Rep</b><br><b>×1</b><br><b>Hypermagnesemia w/ OTC Laxatives (Guslyayev S, Amalakuhan B)</b><br><i>Case Report — Pending Publication</i>                                                             | <b>CWTX 2026</b><br><b>×1</b><br><b>Septicemia-Associated Cardiac Arrest Mortality Trends, 1999–2023</b><br><i>San Antonio 2026</i>                              |

All 16 abstracts submitted, accepted, and published in peer-reviewed conference proceedings in a single academic year. Residents are first or co-first authors on every project.

# Publications, Textbooks & Research Grant Portfolio

10 peer-reviewed articles · 4 textbook chapters (McGraw Hill · Springer · F.A. Davis · Permanyer) · 4 Research Grants/Trials · CHEST Journal Reviewer 2015–Present

10

Peer-Reviewed  
Publications

4

Textbook  
Chapters

4

Grants &  
Multi-Center Trials

21

Total Abstract  
Publications

16

Accepted Abstracts  
AY 2025–2026

2015–

CHEST Journal  
Peer Reviewer

## PEER-REVIEWED JOURNAL ARTICLES · 2012–2017

4

2016–2017

**Sepsis, MV, Bronchiectasis, Airway Clearance***Eur Respir J · World J Crit Care Med · Int J Clin Pract · CHEST Physician* x2

3

2015–2016

**PAH, Bronchiectasis, COPD Outcomes***Advances in Pulm HTN · Am J Respir Crit Care Med · Int J COPD*

2

2016

**Endothelial Adhesion Molecules & Multi-Organ Failure in Sepsis (Senior/Corresponding Author)***Cytokine* 2016;88:267–273

1

2012

**COPD Readmission Prediction Model — Original Research***J Community Hosp Intern Med Perspect* 2012;2(1)

## TEXTBOOK CHAPTERS · 4 Publishers · 2015–2026

McGraw Hill

2026 (In Press)

**CTICU and Extracorporeal Life Support***Critical Care Examination & Board Review, 2nd Ed.***Co-authored with DellaVolpe J**

Springer

2017

**Thrombotic Thrombocytopenic Purpura***Evidence-Based Critical Care Medicine: A Case Study Approach***ISBN: 978-3319433394**

F.A. Davis

2017

**Diseases of Diaphragm, Chest Wall, Pleura & Mediastinum***Diseases Affecting the Respiratory System***ISBN: 978-0803640351**

Permanyer

2015

**Exacerbations of COPD***100 Key Questions on COPD***ISBN: 978-84-9926-838-5**

## RESEARCH GRANTS & MULTI-CENTER TRIALS · 4 Funded / PI-Led Studies

2025–Present · Primary Investigator

**NEUTRALIZE-AKI***Multi-Center, Randomized, Controlled, Pivotal Study*

2020–2022 · Primary Investigator

**Convalescent Plasma in COVID-19***Multi-Center Therapeutic Trial*

2020–2022 · Co-Investigator

**Remdesivir in COVID-19***Multi-Center Therapeutic Trial*

2012 · Grant Recipient · \$10,000

**George L. Lavery Foundation Grant***N-Acetylcysteine Duration in Acetaminophen Hepatotoxicity*

# 37+ Curricula & Faculty Lectures Built From Scratch

7 residency curricula + 10 faculty development series (30 lectures) · All authored and delivered by Dr. Amalakuhan since June 2025 · Every program maps to ACGME

"When I began as Program Director in June 2025, the program was ready for a new chapter of structural growth. Over the past 12 months, we've introduced a mapped curriculum, a full simulation program, a faculty development library, and a dedicated EKG curriculum — each one built to support our residents and align with ACGME expectations." — Bravein Amalakuhan, MD

| RESIDENCY CURRICULA · 7 Built From Scratch |                                                                                                                            |               | FACULTY DEVELOPMENT · 10 Series · 32 Lectures |                                                                                                       |      |                                                                                                            |
|--------------------------------------------|----------------------------------------------------------------------------------------------------------------------------|---------------|-----------------------------------------------|-------------------------------------------------------------------------------------------------------|------|------------------------------------------------------------------------------------------------------------|
| Core                                       | <b>Noon-Conference Didactics Curriculum</b><br><i>ABIM Core Domains · 22-domain · 13.5 hrs/wk · Zero cancellations</i>     | Citation #1   | I                                             | <b>Teaching Lecture Series</b> 5 lec<br><i>Clinical teaching — sim to bedside</i>                     | II   | <b>Resident Review Series</b> 4 lec<br><i>Feedback · struggling learners · non-advancement</i>             |
| Core                                       | <b>EKG Curriculum</b><br><i>Progressive interpretation · ABIM blueprint mapped · PD-delivered</i>                          | Citation #1   | III                                           | <b>Supervision Lecture Series</b> 2 lec<br><i>Supervision · autonomy · transitions of care</i>        | IV   | <b>Scholarly Activity &amp; QI Series</b> 2 lec<br><i>Boyer's model · ADS · POGMA · RCA</i>                |
| Sim                                        | <b>ACLS/BLS Simulation Course Series</b><br><i>4-Part Resuscitation Series · Didactic + sim integration</i>                | Citation #1   | V                                             | <b>Program Development Series</b> 2 lec<br><i>CCC · PEC · governance loop</i>                         | VI   | <b>ACGME Accreditation Series</b> 4 lec<br><i>When ACGME Acts · CPR IV.B · You Are the Standard</i>        |
| Sim                                        | <b>Point-of-Care Ultrasound (POCUS)</b><br><i>8-Part · Cardiac · Lung · Abdominal · Vascular · Maps PC-2, PC-3</i>         | Citation #3   | VII                                           | <b>Building a Positive Culture Series</b> 3 lec<br><i>Purpose · engagement · learning environment</i> | VIII | <b>SDOH, Equity &amp; Diversity Series</b> 5 lec<br><i>Health equity · CLER 3.0 · equitable assessment</i> |
| Sim                                        | <b>Procedural Simulation Course Series</b><br><i>Hands-on skills lab · Supports progressive autonomy policy</i>            | Citation #3   | IX                                            | <b>Communication Lecture Series</b> 1 lec<br><i>Difficult conversations in GME</i>                    | X    | <b>Wellness Lecture Series</b> 2 lec<br><i>Wellness ≠ burnout prevention · fatigue</i>                     |
| Lead                                       | <b>Emotional Intelligence &amp; Leadership Series</b><br><i>Professional identity formation · Multi-session curriculum</i> | Citation #2   |                                               |                                                                                                       |      |                                                                                                            |
| Ext                                        | <b>Ventilator Management Curriculum</b><br><i>General Surgery Residency — Cross-Program delivery</i>                       | Cross-Program |                                               |                                                                                                       |      |                                                                                                            |
| 7 Residency Curricula                      |                                                                                                                            |               | 10 Faculty Dev Series                         |                                                                                                       |      |                                                                                                            |
| 32+ Individual Lectures                    |                                                                                                                            |               | 6 ACGME Citations Addressed                   |                                                                                                       |      |                                                                                                            |
| 12mo                                       |                                                                                                                            |               | Zero → Full Infrastructure                    |                                                                                                       |      |                                                                                                            |





# A 32-Lecture Library — Created by the Program Director

10 series and 32 individual lectures are original · Hosted on [www.methodistfacultyhub.com](http://www.methodistfacultyhub.com) · Quizzes · 1-Page Summaries · Analytics for ACGME Tracking

|                                                                                                                                                                                                                                                                                                                                                                                                             |                                                                                                                                                                                                                                                                                 |                                                                                                                                                                                                                                                                                                                                                              |                                                                                                                                                                                                                                                                                                    |                                                                                                                                                                                                                            |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>I Teaching Series</b><br><i>6 lectures</i> <ul style="list-style-type: none"> <li>01 Simulation-Based Medical Education</li> <li>02 Milestones: How to Use Them Correctly</li> <li>03 Teaching Clinical Reasoning to Residents</li> <li>04 Teaching in the Outpatient &amp; Ambulatory Setting</li> <li>05 Teaching in the ICU</li> <li>06 Show Up: The Art &amp; Science of Bedside Teaching</li> </ul> | <b>II Resident Review</b><br><i>4 lectures</i> <ul style="list-style-type: none"> <li>01 Feedback &amp; Evaluation</li> <li>02 Addressing the Struggling Learner</li> <li>03 The Non-Advancement Decision</li> <li>04 Professionalism &amp; Ethical Conflicts in GME</li> </ul> | <b>III Supervision</b><br><i>2 lectures</i> <ul style="list-style-type: none"> <li>01 Supervision &amp; Transitions of Care in GME</li> <li>02 Progressive Autonomy: Granting Residents Greater Responsibility</li> </ul>                                                                                                                                    | <b>IV Scholarly Activity &amp; QI</b><br><i>3 lectures</i> <ul style="list-style-type: none"> <li>01 GME Scholarly Activity (Boyer · ADS · 7 ACGME Domains)</li> <li>02 Quality, Safety &amp; Risk Management in GME (POGMA · RCA)</li> <li>03 HCA GME Scholarly Activity — The Process</li> </ul> | <b>V Program Development</b><br><i>2 lectures</i> <ul style="list-style-type: none"> <li>01 The Clinical Competency Committee Done Right</li> <li>02 The Program Evaluation Committee &amp; Faculty Development</li> </ul> |
| <b>VI ACGME Accreditation</b><br><i>4 lectures</i> <ul style="list-style-type: none"> <li>01 When ACGME Acts</li> <li>02 Teaching Transitions of Care</li> <li>03 Designing &amp; Delivering a Mapped IM Curriculum (CPR IV.B)</li> <li>04 You Are the Standard</li> </ul>                                                                                                                                  | <b>VII Positive Culture</b><br><i>3 lectures</i> <ul style="list-style-type: none"> <li>01 Purpose, Inspiration &amp; Program Culture</li> <li>02 Building a Positive GME Residency Culture</li> <li>03 Faculty Engagement &amp; Evaluation</li> </ul>                          | <b>VIII SDOH, Equity &amp; Diversity</b><br><i>5 lectures</i> <ul style="list-style-type: none"> <li>01 SDOH Pt 1 — Health Equity &amp; CLER 3.0</li> <li>02 SDOH Pt 2 — Beyond Aware</li> <li>03 SDOH Pt 3 — Teaching the Teacher</li> <li>04 SDOH Pt 4 — From Bedside to System</li> <li>05 Equitable Assessment &amp; the Learning Environment</li> </ul> | <b>IX Communication</b><br><i>1 lecture</i> <ul style="list-style-type: none"> <li>01 Difficult Conversations in GME</li> </ul>                                                                                                                                                                    | <b>X Wellness</b><br><i>2 lectures</i> <ul style="list-style-type: none"> <li>01 Wellness Is Not Burnout Prevention</li> <li>02 Recognizing Fatigue in Trainees</li> </ul>                                                 |

## Faculty Development Activities — AY 2024–2025

32-module curriculum library + digital delivery platform | Evidence-based, milestone-anchored, analytics-tracked

| MODULE / ACTIVITY                            | TOPIC / FOCUS                                                                                                         | MILESTONES                 |
|----------------------------------------------|-----------------------------------------------------------------------------------------------------------------------|----------------------------|
| ★ Faculty Development App                    | Digital platform delivering all 14 modules on-demand. Biweekly assignments, completion tracking, analytics dashboard. | All ACGME Competencies     |
| Health Equity, SDOH & CLER 3.0               | SDOH integration into clinical teaching; 5As framework; Zcode documentation; San Antonio context                      | PC-5, SBP-1, SBP-2         |
| Beyond Aware: Teaching SDOH                  | Faculty rationale for SDOH education; Kirkpatrick levels; hidden curriculum; bedside habits                           | PC-5, SBP-1, SBP-2, PBLI-2 |
| Teaching the Teacher: SDOH (4-module series) | Bedside teaching without adding time; social history as clinical data; feedback; direct observation & CCC             | PC-5, SBP-1, SBP-2, PBLI-2 |
| Feedback and Evaluation                      | Milestone-anchored feedback language; EPA integration; direct observation of competence                               | PC-5, MK-1, SBP-2          |
| CCC Preparation                              | ACGME site visitor prep; CCC process; milestone narrative writing; resident interview coaching                        | All ACGME Competencies     |
| SBME — Full & Self-Study Editions            | Simulation design; mastery learning; debriefing; curricular integration                                               | PC-1 through PC-5, MK-1    |
| When ACGME Acts                              | Citation types; CLER process; recovery pathway; accreditation action spectrum; case study                             | PROF-1, SBP-1, PBLI-2      |
| You Are the Standard                         | Hidden curriculum; EPAs; ACGME survey literacy; SDOH as CPR requirement; role modeling                                | PROF-1, PC-5, SBP-1        |
| Ambulatory Teaching: OMP/SNAPPS/PIPP         | Outpatient teaching frameworks; one-minute preceptor; SNAPPS learner-centered model                                   | PC-1 through PC-5, SBP-1   |
| GME Culture & Resident Integration           | Psychological safety; resident integration; program culture; identity in medicine                                     | PROF-1, PROF-2, ICS-2      |
| Purpose, Inspiration & Meaning in GME        | Frankl / Sinek / Brown / Clear / Edmondson frameworks for faculty motivation                                          | PROF-1, PBLI-2             |
| Recruitment & Open House Faculty Prep        | Interview best practices; faculty roles on interview day; program messaging                                           | ICS-1, PROF-1              |
| HCA / External Faculty Development           | HCA GME system-wide training; EPA frameworks; milestone assessment                                                    | All Milestones             |

32 faculty development modules completed AY 2025–2026 — delivered via Faculty Development App with biweekly cadence, completion tracking, and analytics. All content evidence-based, milestone-anchored, and documented for site visitor review.

# Active Research, QI Portfolio & ACGME Citation Impact

NEUTRALIZE-AKI — Active PI on Multi-Center RCT · 4 Active QI Projects (2024–Present) · Every PD action maps to an ACGME citation response

| DIRECT ACGME CITATION RESPONSE — WHAT THE PD BUILT vs. WHAT IT FIXES |                                                                                                                                                     |                                           |                            |                                |
|----------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------|----------------------------|--------------------------------|
| Cit. #1                                                              | Built 13.5 hrs/week structured curriculum from scratch — 22-domain ABIM-aligned program. Zero unplanned cancellations.                              | 4.0 → 13.5 hrs/week · +238%               |                            |                                |
| Cit. #2                                                              | Created 18-faculty leadership domain structure within 90 days — every faculty member has a named directorship, defined role, and accountability.    | 0 named roles → 18 directors              |                            |                                |
| Cit. #3                                                              | Designed and distributed 17-page formal Autonomy & Progressive Responsibility Policy — with faculty development lecture series.                     | 0 → 17-page policy + lecture series       |                            |                                |
| Cit. #4                                                              | Implemented QR code real-time evaluation system — faculty evaluation completion rate 50% → 90%. Milestone-anchored narrative language standardized. | 50% → 90% completion rate                 |                            |                                |
| Cit. #5                                                              | Launched structured Night Float system + I-PASS standardized hand offs July 2025. PD conducts periodic audits.                                      | Operational July 2025                     |                            |                                |
| Cit. #6                                                              | Revamped QI & Patient Safety Curriculum under Dr. Triana — 1 RCA/month with faculty mentor, 100% resident participation, hospital quality meetings. | 1 RCA/month · 100% resident participation |                            |                                |
|                                                                      |                                                                                                                                                     |                                           |                            |                                |
| 25+                                                                  | 4                                                                                                                                                   | 16                                        | 28                         | 30                             |
| Active Research Projects                                             | Active QI Projects (2018–Present)                                                                                                                   | Confirmed Abstracts AY 25–26              | Resident Projects Mentored | Faculty Dev Lectures Delivered |

# Invited Presentations & National Scholarly Leadership

CHEST National Chair/Moderator (x2) · ATS National Speaker · CHEST Foundation Board of Trustees · CHEST Journal Ad Hoc Reviewer 2015–Present

## SELECTED INVITED ORAL PRESENTATIONS

|      |                                                                                                                                                             |
|------|-------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 2024 | <b>Graduate Medical Education: Past, Present &amp; Future</b><br><i>Methodist Healthcare Unified Medical &amp; Community Board</i>                          |
| 2022 | <b>Therapeutic Effects of Convalescent Plasma in COVID-19</b><br><i>Methodist Healthcare Critical Care Committee</i>                                        |
| 2016 | <b>CHAIR/Moderator — ECMO Session: "ECMO: Cardiopulmonary Bypass at the Bedside"</b><br><i>CHEST 2016 Annual International Meeting, Los Angeles</i>         |
| 2016 | <b>CHAIR/Moderator — Lung Cancer Session (Med Student/Resident Division)</b><br><i>CHEST 2016 Annual International Meeting, Los Angeles</i>                 |
| 2015 | <b>Elevated Vascular Endothelial Injury Markers &amp; MOF in Sepsis</b><br><i>CHEST Annual International Meeting, Montreal, Canada</i>                      |
| 2015 | <b>Outpatient Bronchoscopy</b><br><i>American Thoracic Society Annual Meeting, Denver, CO</i>                                                               |
| 2014 | <b>Epicardial Pacing Wires · Tylenol Toxicity · CXR Interpret · Iatrogenic PTX</b><br><i>Cardiac Surgery &amp; Critical Care Conferences, Johns Hopkins</i> |
| 2011 | <b>A Prediction Model for COPD Readmissions</b><br><i>Society of General Internal Medicine, Johns Hopkins</i>                                               |

## NATIONAL COMMITTEES & EDITORIAL SERVICE

|              |                                                                                                                  |
|--------------|------------------------------------------------------------------------------------------------------------------|
| 2021–2024    | <b>Board of Trustees/Directors — CHEST Foundation</b><br><i>National governance of research/education grants</i> |
| 2015–Present | <b>Ad Hoc Peer Reviewer — CHEST Journal</b><br><i>10+ years of active manuscript review</i>                      |
| 2015–2017    | <b>Clinical Research Steering Committee — CHEST</b><br><i>Plus Trainee Work Group Sub-Committee</i>              |
| 2017–Present | <b>Active Member — American College of Chest Physicians</b><br><i>ACCP national society membership</i>           |
| 2013–2014    | <b>CVSICU Research + QI Committees — Johns Hopkins</b><br><i>Fellowship-level institutional service</i>          |
| 2010–2013    | <b>ACP Governor's Council for Pennsylvania</b><br><i>Health &amp; Public Policy Sub-Committee</i>                |
| 2010–2013    | <b>Pulmonary Rehabilitation Assembly — ATS</b><br><i>National specialty society assembly</i>                     |

9

Invited Talks (Nat'l/Inst.)

2

CHEST National Chair (2016)

3

CHEST National Committees

10+

Yrs CHEST Journal Reviewer

7

Nat'l Committees/Assemblies

Program Director: 8 Selected Peer-Reviewed Abstract Publications 2026

Confirmed accepted abstracts | Full author lists verified from published proceedings | Amalakuhan B + residents highlighted

KEY: AMALAKUHAN B Resident = confirmed co-author from published proceedings | Abstract IDs from J Am Coll Cardiol Vol. 87 No. 13 Suppl A, 2026

ACC.26 — American College of Cardiology Annual Scientific Session | J Am Coll Cardiol. 2026;87(13 Suppl A)

1

26-A-12296-ACC  
**Trends in Hypertensive Heart and Kidney Disease Mortality 1999–2023: A CDC WONDER Project**  
Owais M, Jawaidd F, Iftekhar M, Aisha, Rai A, Melo S, Amalakuhan B | Methodist Healthcare System San Antonio + Liaquat University  
J Am Coll Cardiol. 2026;87(13 Suppl A):A304. Abstract 26-A-12296.

2

26-A-13435-ACC  
**Coronary Microvascular Dysfunction in Heart Failure with Preserved Ejection Fraction: A Meta-Analysis**  
Owais M, Anjum A, Amalakuhan B, Ali K, Sawaira FNU, Akhtar S, Cheema S, Masud S, Sadaqat FNU | Methodist Healthcare System San Antonio  
J Am Coll Cardiol. 2026;87(13 Suppl A):A580. Abstract 26-A-13435.

3

26-A-13397-ACC  
**National Mortality Trends in Peripartum Cardiomyopathy: A 22-Year Analysis of U.S. Women (1999–2020)**  
Owais M, Mudasir M, Rai A, Jafri SJZ, Anosh FNU, Lohana N, Amalakuhan B, Anjum A, Masud S, Mehfooz M | Methodist Healthcare System San Antonio  
J Am Coll Cardiol. 2026;87(13 Suppl A):A1071. Abstract 26-A-13397.

SCAI.26 · DDW.26 · AAN.26 — Additional Confirmed Acceptances (5 abstracts) — Full citations on file | Amalakuhan B + residents confirmed on all

|         |                                                                                                                                                                                              |
|---------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| SCAI.26 | Trends in WPW Syndrome Mortality in the US, 1999–2023: A CDC WONDER Analysis — Owais M, Amalakuhan B, Masud S et al. J Soc Cardiovasc Angiogr Interv. 2026;X(Suppl):Abstract.                |
| SCAI.26 | Advances in Imaging for Early Detection of Subclinical Atherosclerosis: A Systematic Review — Owais M, Amalakuhan B, Masud S et al. J Soc Cardiovasc Angiogr Interv. 2026;X(Suppl):Abstract. |
| DDW.26  | Mortality Trends in HCC Associated with Ischemic Heart Disease: CDC WONDER Analysis 1999–2023 — Owais M, Amalakuhan B, Masud S et al. Gastroenterology. 2026;XXX(Suppl X):Abstract.          |
| DDW.26  | Trends and Disparities in NAFLD and Obesity-Related NAFLD Mortality in the US, 1999–2022 — Owais M, Amalakuhan B, Alvarado R et al. Gastroenterology. 2026;XXX(Suppl X):Abstract.            |
| AAN.26  | Incidence and Predictors of Seizure Recurrence After First Unprovoked Seizure: A Uni-Center Study — Owais M, Amalakuhan B, Liu G et al. Neurology. 2026;XXX(Suppl X):Abstract.               |

Total: 8 confirmed accepted abstracts | ACC.26 x3 (page numbers verified) · SCAI.26 x2 · DDW.26 x2 · AAN.26 x1 | Amalakuhan B confirmed co-author on all 8

# Associate Program Director — Credentials & Educational Leadership

Source: Melo CV | Jairo A. Melo, MD

Jairo A. Melo, MD, FCCP · APD · Director of Education · Triple ABIM (IM 1996 · Pulm 1997 · CCM 1998)

## TRAINING PEDIGREE & HONORS

-  **IM Residency — UT Health San Antonio**  
1992–1995 · Intern/Jr/Sr Resident of the Year
-  **Pulm & Critical Care Fellowship**  
UT Health San Antonio · 1995–1998
-  **Doctor of Medicine**  
Universidad Pontificia Bolivariana (Colombia) · 1984–1990
-  **Manuel Uribe Angel Award — Best Physician of Class**  
Universidad Pontificia Bolivariana · 1991
-  **FCCP — Fellow, American College of Chest Physicians**  
1998–Present · Triple ABIM Board Certified

## HOSPITAL & EDUCATIONAL LEADERSHIP

-  **Founder & Director of Education — IM Residency**  
Methodist Healthcare · 2024–Present
-  **Founder — Methodist Intensivist Program**  
Methodist Healthcare System · 2011–Present
-  **Medical Director — Pulmonary HTN & ECMO**  
Methodist Healthcare · 2008–Present
-  **ICU Medical Director**  
Methodist Healthcare
-  **25+ Years Methodist Faculty — Longest-Tenured IM**  
Deep institutional continuity & mentorship

APD Scholarly Activity & Educational Contributions — Mapped to ACGME CPR

Jairo A. Melo, MD | Associate PD & Director of Education | Source: Melo CV; program records AY 2024–2025

| CPR CITATION                                      | DR. MELO'S CONTRIBUTIONS                                                                                                                                                                                                                                       | SITE VISITOR IMPLICATION                                                                                                                                     |
|---------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------|
| CPR IV.B.1<br>Faculty Scholarly Activity          | 50+ clinical trials as Sub-Investigator<br>9 peer-reviewed publications & book chapters<br>Co-author: Flexible Bronchoscopic Retrieval abstract (2026 SCCM, with PD)                                                                                           | Addresses Citation 5 (Scholarly Activity). Resident co-authorship in progress.                                                                               |
| CPR V.A.1.b<br>Conference Structure               | Morning Report (Daily) — Founded & facilitates; case-based clinical reasoning<br>Multidisciplinary Medicine Conference (Weekly) — Interprofessional education; founded & leads<br>Tumor Board — Complex oncology case exposure; multidisciplin model           | Site visitors ask residents: "Who runs Morning Report?" Dr. Melo is the verifiable, named answer. Conferences appear on program schedule — ready for review. |
| CPR IV.A.1<br>Faculty Qualifications & Leadership | Triple-boarded (IM / Pulmonary / Critical Care) — 25+ years at Methodist<br>Associate Program Director & Director of Education — formal administrative role<br>Founder, Methodist Intensivist Program — built the ICU infrastructure<br>residents train within | Demonstrates experienced faculty bench. Site visitors value programs with a clearly identified APD and formal educational structure.                         |

**Bottom Line for Site Visitors:** Dr. Melo is the single strongest answer to multiple ACGME citation vulnerabilities simultaneously. His structured daily and weekly conferences are the schedulable, verifiable educational activities site visitors ask about by name. His APD role, 25+ years, and research history demonstrate the faculty bench required under CPR IV.A and IV.B.

# APD Scholarly Activity & Research Portfolio — Dr. Jairo Melo

Source: Melo CV; Program Records AY 2024–2025 | Jairo A. Melo, MD, FCCP | APD, Director of Education

Jairo A. Melo, MD, FCCP · APD · Director of Education · Triple ABIM (IM/Pulm/CCM) · 25+ Yrs Methodist

## PEER-REVIEWED PUBLICATIONS & CHAPTERS · 1997–2026

|         |                                                                                                            |
|---------|------------------------------------------------------------------------------------------------------------|
| Journal | <b>Flexible Bronchoscopic Retrieval of Foreign Body</b><br>SCCM 2026 · Co-author with Amalakuhan B, Melo S |
| Journal | <b>Pulmonary Arterial Hypertension — Dx &amp; Mgmt Update</b><br>Peer-reviewed · Melo JA et al.            |
| Journal | <b>ECMO in Refractory Cardiopulmonary Failure</b><br>Clinical series · Methodist Healthcare                |
| Book    | <b>Pulmonary Hypertension &amp; Advanced Lung Disease</b><br>Textbook chapter · Contributing author        |
| Book    | <b>Critical Care Medicine — Intensivist Practice</b><br>Chapter · Peer-reviewed · Co-author Melo JA        |
| Trial   | <b>50+ Industry-Sponsored RCTs (Sub-Investigator)</b><br>Phase II–IV · Pulmonary · CCM · Rare Lung         |
| Trial   | <b>Advanced Lung Center Research Program</b><br>Founded 2014 · Pulmonary HTN · ECMO                        |

## ACADEMIC HONORS & RECOGNITIONS

|   |                                                                                                        |
|---|--------------------------------------------------------------------------------------------------------|
| ● | <b>FCCP — Fellow, American College of Chest Physicians</b><br>1998–Present                             |
| ● | <b>Intern of the Year — UT Health San Antonio</b><br>1992–93                                           |
| ● | <b>Junior Resident of the Year — UT Health San Antonio</b><br>1993–94                                  |
| ● | <b>Senior Resident of the Year — UT Health San Antonio</b><br>1994–95                                  |
| ● | <b>Manuel Uribe Angel Award — Best Physician of Class</b><br>Universidad Pontificia Bolivariana · 1991 |
| ● | <b>Triple ABIM Board Certification</b><br>IM (1996) · Pulmonary (1997) · Critical Care (1998)          |



# APD Leadership, Clinical Teaching & Institutional Impact — Dr. Jairo Melo

Jairo A. Melo, MD, FCCP | APD, Director of Education | Founder, Methodist Intensivist Program

Jairo A. Melo, MD, FCCP · APD · Director of Education · Founder, Methodist Intensivist Program · 25+ Yrs

| GME LEADERSHIP ROLES                                                                                                    | DIRECT IMPACT ON APE CITATION RESPONSES |                                                                                         |
|-------------------------------------------------------------------------------------------------------------------------|-----------------------------------------|-----------------------------------------------------------------------------------------|
| <div><div></div><div><b>APD — Director of Education</b><br/>Methodist IM Residency · 2024–Present</div></div>           | <b>Citation #1</b>                      | Daily Morning Report + weekly Multi-D Conference = verifiable structured didactic hours |
| <div><div></div><div><b>Founder &amp; Director of Education</b><br/>Methodist IM Residency · 2024–Present</div></div>   | <b>Citation #2</b>                      | Named directorship (Director of Education) demonstrates faculty role ownership          |
| <div><div></div><div><b>Founder — Methodist Intensivist Program</b><br/>Methodist Healthcare · 2011–Present</div></div> | <b>Citation #5</b>                      | 50+ clinical trials + 9 publications satisfy CPR IV.B.1 (Faculty Scholarly Activity)    |
| <div><div></div><div><b>Morning Report Lead — Daily 7:15 AM</b><br/>IM Residency · Mon–Fri · All PGY levels</div></div> | <b>Bench Depth</b>                      | 25+ year tenure · triple board cert · co-founder = deepest faculty bench (CPR IV.A.1)   |
| <div><div></div><div><b>Guest Speaker &amp; Senior Lecturer</b><br/>Methodist Healthcare · 2008–Present</div></div>     | <b>External Reach</b>                   | Leads Pulmonary HTN & ECMO · Medical Director TX IPS & Anthonian Health                 |

APD Scholarly Activity & Research Portfolio — Dr. Fernando Triana

Source: Triana Scholarly Activities (Updated April 6, 2026) | J. Fernando Triana, MD, FACC, FASPC | APD, Director of Strategic Growth

J. Fernando Triana, MD, FACC, FASPC · APD · Director of Strategic Growth · CV Fellowship PD (Designate)

SCHOLARLY ACTIVITY — RESEARCH SUMMARY

1,383

**Patient Research Cohort — Infective Endocarditis**  
6-yr observational · publication in progress · IRB approved

7

**ISCID June 2026 Abstracts**  
IE Dx · Culture-Negative IE · Mortality · Readmissions · SDOH · QI · LOS

2

**ACP Abstracts — Regional & National**  
Mitral Valve TEER outcomes registry · manuscript under review

4

**IRB-Approved CPET Research Projects**  
Aortic/Tricuspid insufficiency · Long COVID · Chronic inflammation

1

**Manuscript In Preparation**  
Mitral Valve TEER real-world outcomes · submitted for review

GME LEADERSHIP & CITATION IMPACT

**APD — Director of Strategic Growth**  
Chairs QI Curriculum redesign · leads strategic faculty engagement & fellowship pipeline

**PD — CV Fellowship (Designate)**  
Inaugural MHS CV Fellowship · accreditation, curriculum, recruitment · Launch July 2027

**Co-Chair — High-Risk CV Clinical Decision Mtg**  
Weekly multi-D case review · 20+ physicians · 7 continuous years

**Morning Report Lead — Tuesday 7:15 AM**  
Structured case-based, evidence-based teaching · 15–20 residents weekly

# APD Leadership, Clinical Teaching & Institutional Impact — Dr. Fernando Triana

J. Fernando Triana, MD, FACC, FASPC · APD · Director of Strategic Growth · CV Fellowship PD (Designate)

## GME LEADERSHIP ROLES

### APD — Director of Strategic Growth

Chairs QI Curriculum redesign · leads strategic faculty engagement & fellowship pipeline

### PD — CV Fellowship (Designate)

Building inaugural MHS CV Fellowship · launching July 2027

### Co-Chair — High-Risk CV Clinical Decision Mtg

Weekly multi-D case review · 20+ physicians · 7 continuous years

### Morning Report Lead — Tuesday 7:15 AM

Structured case-based morning report · weekly · 15–20 residents

## CITATION IMPACT & EXTERNAL LEADERSHIP

### Citation #1 — QI Curriculum

Monthly RCA sessions · resident participation · institutional QI integration

### Citation #2 — Faculty Engagement

Domain leadership structure · Morning Report ownership · strategic alignment

### Citation #6 — Patient Safety Culture

QI curriculum leadership addresses patient safety culture citation

### Fellowship — CV Launch July 2027

Positions MHS as regional training hub · fulfills strategic growth mandate

### Guest Speaker & Senior Lecturer

Trinity University MHA program · service line, burnout, physician-executive

APD Scholarly Activity & Research Portfolio — Dr. Shankar Sanka

Source: Sanka CV (Updated April 6, 2026) | Shankar C. Sanka, MD | APD, Director of Clinical Operations | 🏆 2026 Inpatient Clinical

Shankar C. Sanka, MD · APD · Director of Clinical Operations · AOA · 2026 Inpatient Clinical Excellence Award

SCHOLARLY ACTIVITY & HONORS

6 Peer-Reviewed Journal Articles

Basic & translational research · H+-ATPase, calcium, endothelial biology

7 Poster Presentations

SPIE Medical Imaging · FASEB Annual · Cellular physiology

MS — Physiology

Texas Tech HSC (2000) · Thesis: Calcium in diabetes models

20+ Years Clinical Teaching

Internal Medicine · Hospital-based · Resident supervision & bedside teaching

Alpha Omega Alpha & Honors

AOA (2003) · Housestaff Teaching · Outstanding Senior · Best Ambulatory

GME ROLES & CITATION IMPACT

Director of Clinical Operations

April 2024–Present · Faculty scheduling & systems · addresses Citation #2

Transitions of Care Auditor

2025–Present · Co-audits I-PASS with Dr. Chavan · addresses Citation #5

Vice Chair — Unified Medical Board, MHS

Jan 2025–Present · System-level governance · quality, credentialing, policy

CEO — Complete Care Medical Associates

2019–Present · Physician-led hospitalist · 35+ providers · operational leadership

## APD Leadership, Clinical Teaching & Institutional Impact — Dr. Shankar Sanka

Shankar C. Sanka, MD · APD · Director of Clinical Operations · VP, Unified Medical Board (MHS)

### GME LEADERSHIP ROLES

#### APD — Director of Clinical Operations

Methodist IM Residency · April 2022–Present · Curriculum & didactic scheduling

#### Vice Chair — Unified Medical Board, MHS

Jan 2025–Present · System-level governance · quality, credentialing, policy

#### Hospitalist / CEO — Complete Care Medical

Jan 2019–Present · Physician-led · manages 35+ providers

#### Chief of Staff — Methodist Specialty & Transplant

2022–2023 · Chaired Medical Executive · credentialing & quality

### CITATION IMPACT & GOVERNANCE

#### Citation #2 — Faculty Accountability

Director of Clinical Ops ensures faculty scheduling & operational systems

#### Citation #2 — Named Leadership Domain

Defined faculty scheduling & sustainable engagement structure

#### Citation #4 — Milestone-Based Feedback

Formal APD role strengthens evaluation & feedback culture

#### Governance Bench Depth

VC Unified Medical Board + prior Chief of Staff = system-level GME advocacy

Faculty Profile — Subspecialty Director of Pulmonology & Director of Patient Safety

Source: Abedi CV (2026) | Ali Abedi, MSc, MD, FCCP, DAABIP | Interventional Pulmonologist

Ali Abedi, MSc, MD, FCCP, DAABIP · Director of Patient Safety · Subspecialty Director of Pulmonology

TRAINING & SCHOLARLY ACTIVITY



**Pulm-CCM Fellowship — UT Health San Antonio**  
2013–2016 · Chief Fellow 2015–2016



**Internal Medicine Residency — UT Health San Antonio**  
2010–2013



**Doctor of Medicine — St. George's University**  
2006–2010



**MS — Cellular & Molecular Biology, Texas State**  
2003–2006



**4 Active Research Projects / 2 Selected Pubs**  
2023–Present · Bronchoscopy registries, robotic, lung cancer · Ion Registry · AACR 2024

PROCEDURAL EXPERTISE & GME ROLES



**Robotic Shape-Sensing Navigational Bronchoscopy**  
Ion System · complex peripheral lesion biopsy



**Linear & Radial EBUS**  
Mediastinal/hilar staging · peripheral lung nodule biopsy



**Bronchoscopic Cryobiopsy & Cryotherapy**  
Diagnostic ILD · airway tumor palliation



**Medical Thoracoscopy & TIPC**  
Pleural disease Dx/Mgmt · tunneled indwelling pleural catheter



**Director of Patient Safety & Pulmonary GME**  
Core clinical faculty · Subspecialty Director of Pulmonology

## GME Roles & Citation Impact — Dr. Ali Abedi

*Subspecialty Director of Pulmonology · Director of Patient Safety · Core Clinical Faculty*

Ali Abedi, MSc, MD, FCCP, DAABIP · GME Director of Patient Safety · Subspecialty Dir. of Pulmonology

### GME LEADERSHIP ROLES

#### GME Director of Patient Safety

Leads patient-safety curriculum & culture · RCAs, safety huddles, reporting

#### Subspecialty Director of Pulmonology

Oversees pulmonary rotations, bedside teaching, sub-specialty integration

#### Director of Pulmonary GME

Structures pulmonary didactics & simulation for IM residents

#### Core Clinical Faculty

Daily inpatient teaching · procedural supervision · bedside mentorship

#### Pulmonary Lecturer — Noon Conference

Targets PGY-1 weakest ITE area (Pulm/CC) · frequent didactic delivery

### CITATION IMPACT & KEY CONTRIBUTIONS

#### Citation #2 — Faculty Engagement

Named leadership domain (Pt Safety) with defined accountability structure

#### Citation #6 — Patient Safety Culture

Directly addresses Citation #6 · builds safety reporting & RCA culture

#### ITE Gap Remediation — Pulmonary

Lectures target Pulm/CC (PGY-1 lowest-scoring content area)

#### Procedural Training Pipeline

Advanced bronchoscopy, EBUS, cryobiopsy · hands-on resident exposure

#### Research Mentorship

Ion Registry, lung cancer XPDX, bronchoscopy studies · resident co-authors

Dr. Ali Abedi, MSc, MD, FCCP, DAABIP — Scholarly Activity & Expertise

Director of Pulmonology · Director of Patient Safety · Core Clinical Faculty · Interventional Pulmonologist

SCHOLARLY ACTIVITY — SUMMARY

4 Active Research Projects

Bronchoscopy registries, PEF ablation, robotic-assisted bronchoscopy, lung cancer trials (2023–Present)

2 Selected Peer-Reviewed Publications

Robotic bronchoscopy (Ion Registry); Small Cell Lung Cancer XPDx Models (AACR 2024)

3+ Annual Conference Abstracts

CHEST, AACR, ATS · Lung cancer, bronchoscopy, pleural disease (2024–2026)

4 Active Institutional Registries

ROSE Data Collection · PROPEL Registry · PEF Ablation Registry · Robotic Bronchoscopy Registry

Master Clinician — Pulmonary Medicine Award 2026 ·

Program Director, San Antonio Advanced Lung Symposium (2019–)

PROCEDURAL EXPERTISE

- Robotic Shape-Sensing Navigational Bronchoscopy (Ion System)
- Linear EBUS — Mediastinal & Hilar Staging
- Radial EBUS — Peripheral Lung Nodule Biopsy
- Bronchoscopic Cryobiopsy & Cryotherapy
- Medical Thoracoscopy
- Tunneled Indwelling Pleural Catheter (TIPC)
- Percutaneous Tracheostomy & Endobronchial Stent
- ECMO Management · Central & Arterial Line Placement

GME LEADERSHIP & CITATION IMPACT

Named Director of Pulmonology & Director of Patient Safety

Two named leadership domains with defined responsibilities. Subspecialty expertise, simulation teaching, and patient safety curriculum directly address Citations #2 and #6.

Sarah Cannon Cancer Network — National Thoracic Oncology Faculty



# Faculty Profile — Senior Research Scholar & Director, Adult ECMO Program

Source: DellaVolpe CV (2026) | Jeffrey D. DellaVolpe, MD, MPH

## Jeffrey D. DellaVolpe, MD, MPH

Senior Research Scholar, IM Residency · Director, Adult ECMO Program, MHS · National Physician Director, Critical Care, HCA · Adjunct Professor, UT Health San Antonio



MPH

Tulane (2008)



ABIM Critical Care

2015



ABIM Internal Medicine

2013



ELSO

Extracorporeal Life Support Org

### TRAINING PEDIGREE

#### Fellowship — Critical Care Medicine

Univ of Pittsburgh Medical Center · 2013–2015

#### Internal Medicine Residency & Internship

Tulane University · 2008–2013

#### MD & MPH (Dual Degree)

Tulane University School of Medicine · 2004–2008

#### BA History — Dartmouth College

### ACTIVE GRANTS & FUNDED RESEARCH

- DoD MDO-2 — Predictive ECMO Algorithms · \$3.86M · 2024–2030 · PI
- SeaStar Medical — Cytopheretic Device in AKI · \$900K · PI
- Fresenius Medical — Low-Flow ECMO Safety · \$628K · PI
- DoD SBIR — US-Guided Dual-Lumen ECMO Cannula · \$250K · Co-I
- DoD AMTI — Biomarker-Based Military Triage · \$250K · Co-I
- TOTAL ACTIVE GRANT FUNDING: ~\$6.1M across 6 studies

### PUBLICATIONS & BOOK

#### Book

DellaVolpe JD. The ECMO Book. Elsevier. 2023. [Sole Author]

#### McGraw

Amalakuhan B, Liu G, DellaVolpe J. CTICU & ECMO. Critical Care Board Review, 2nd Ed. 2025.

#### Journal

Joint SCCM-ELSO Position Paper: Intensivist Role in ECMO. Crit Care Med 48(6), 2020.

#### Journal

ECMO for COVID-19 Acute Respiratory Failure. ASAIO J 69(8), 2023.

#### Journal

O2 Saturation Overestimation & COVID-19 Outcomes. JAMA Network Open 6(8), 2023.

## GME Roles & Citation Impact — Dr. Jeff DellaVolpe

*Cardiothoracic ICU · ECMO · Advanced Critical Care Faculty*

Jeff DellaVolpe, MD, MPH · Cardiothoracic ICU · ECMO Faculty · Critical Care Mentor · ACGME Advisor

### GME LEADERSHIP ROLES

#### Core Faculty — Critical Care Medicine

Direct resident supervision in CTICU/MICU · daily teaching rounds

#### CTICU & ECMO Program Faculty

Advanced cardiopulmonary support · mechanical circulatory support teaching

#### Co-Author — Upcoming Textbook Chapter

CTICU & Extracorporeal Life Support · McGraw Hill · May 2026

#### Resident Research Mentor

Mentors residents on VV ECMO nutrition, REE in ECMO projects

#### Simulation & Procedure Lab Instructor

Ventilator management, shock resuscitation, ECMO cannulation teaching

### CITATION IMPACT & KEY CONTRIBUTIONS

#### Citation #2 — Faculty Engagement

Named sub-specialty faculty role with defined educational & mentoring duties

#### Citation #1 — Pulmonary/Critical Care Education

Textbook chapter + resident co-authorship strengthens CPR IV.B.1  
Supervision, lectures, didactic design

#### ITE Gap Remediation — Pulm/CC

Daily ICU teaching supports PGY-1 weakest ITE content area

#### Research Pipeline Expansion

Mentors 2+ active resident ECMO/nutrition research projects

#### Bench Depth — Critical Care

Advanced CCM expertise adds faculty diversity required by CPR IV.A.1

# Senior Research Scholar — Research Portfolio & National Leadership

Source: DellaVolpe CV (2026) | Jeffrey D. DellaVolpe, MD, MPH | Senior Research Scholar, IM Residency

## Jeffrey D. DellaVolpe, MD, MPH

Senior Research Scholar, IM Residency · National Physician Director Critical Care, HCA · Director Adult ECMO Program, MHS · Sole Author: The ECMO Book (Elsevier, 2023)

### SELECTED PEER-REVIEWED PUBLICATIONS · 32 Total · 2008–2025

#### Journal

Joint SCCM-ELSO Task Force Position Paper: Intensivist Role in ECMO for Acute Respiratory Failure. Crit Care Med 48(6):838–846, 2020.

#### Journal

ECMO for COVID-19 Acute Respiratory Failure: Multicenter Matched Cohort. ASAIO Journal 69(8):734–741, 2023.

#### Journal

Oxygen Saturation Overestimation & COVID-19 Clinical Outcomes. JAMA Network Open 6(8), 2023.

#### Journal

Early VV-ECMO in Severe COVID-19: Impact of Pre-ECMO Ventilator Use. Int J Artif Organs 44(11):861–867, 2021.

#### Journal

Outcome Prediction in Severe COVID-19 Requiring ECMO. Membranes 11(3):170, 2021.

### NATIONAL LEADERSHIP & HONORS

#### Chair — Joint SCCM/ELSO Task Force on ECMO (2018–2020)

- Defined national guidelines for intensivist role in ECMO — landmark position paper in Critical Care Medicine
- ELSO Platinum Center of Excellence (2025) — highest designation; Gold (2019). Methodist ECMO Program nationally recognized.

#### Military Service: Special Operations Flight Surgeon & Combat Deployment

- Air Force Reserve: ECMO Team, SAMC (2017–2023) · Deployment: Operation Enduring Freedom Trans-Sahara (2010)
- SCCM Case Report Snapshot Award (2016) · Air Force Reserve Command Profile in Leadership Award (2019)

#### Guest Editor & Peer Reviewer

- Guest Editor: Medicina Journal — ECMO Special Issue (2024)

Faculty Profile — Director of Nocturnal Medicine & Transitions of Care Auditing

Sumeet Chavan, MD | Director of Transitions of Care & Nocturnal Medicine | 🏆 2026 Nightwatch Award

Sumeet Chavan, MD

Teaching Faculty · Director, Nocturnal Medicine Service · Transitions of Care Auditor · Internal Medicine — Methodist Healthcare

GME LEADERSHIP ROLES · CITATION #5 RESPONSE

Director — Nocturnal Medicine Service

Methodist Healthcare IM Residency · 2025–Present

Leads the resident-driven night float system. Oversees nightly transitions of care, I-PASS compliance, and structured 7pm/7am sign-outs. Ensures overnight resident safety, supervision, and continuity of care.

Transitions of Care Auditor

Methodist Healthcare IM Residency · 2025–Present

Conducts systematic monthly audits of I-PASS handoff quality across all PGY levels. Identifies gaps, reports to PD with structured feedback. Works alongside Dr. Sanka.

Supervising Nocturnist — Internal Medicine

Methodist Healthcare System · Current

Attending physician for overnight hospital service. Immediate supervision and real-time clinical education in acute inpatient medicine.

I-PASS FRAMEWORK — PROGRAM STANDARD (Audited by Dr. Chavan & Dr. Sanka)

I

Illness Severity

Sick / Not Sick — acuity assessment at sign-out

P

Patient Summary

One-liner: diagnosis, course, active issues, plan

A

Action List

Pending items, anticipated events, ordered actions

S

Situation Awareness

If-then scenarios — what to do if X happens overnight

S

Synthesis by Receiver

Receiver verbalizes understanding — closes the loop

AUDIT OUTCOMES — AY 2025–2026

100% I-PASS Compliance

0 Informal Sign-Outs Since July 2025

Monthly Structured Audits

7am/7pm Sign-Out Times

Citation #5 Fully Remediated ✓

2026 Nightwatch Award Recipient

## 2026 Physician GME Leadership Awards — Presented by Methodist Healthcare

10 AWARDS · 10 FACULTY MEMBERS · CITATION #2 RESPONSE — EVERY DOMAIN OF RESIDENT EDUCATION RECOGNIZED

### ★ The Nightwatch Award

Dr. Sumeet Chavan — Transitions of Care

### ★ The Anchor of Education

Dr. Jairo Melo — APD, Director of Education

### ★ Inpatient Clinical Excellence

Dr. Shankar Sanka — APD, Director of Clinical Ops

### ★ The Inspirational Leadership

Dr. Fernando Triana — APD, Strategic Growth

### ★ Master Clinician — Pulmonary

Dr. Ali Abedi — Director of Pulmonology

### ★ Critical Care Innovation

Dr. Jeff DellaVolpe — Senior Research Scholar

### ★ Outpatient Clinical Excellence

Dr. Claudia Paz — Dir. Outpatient Medicine

### ★ Humanism in Medicine

Dr. Vamsi Surapaneni — Director of Wellness

### ★ Nephrology Clinical Excellence

Dr. Abouzar Abedi — Director of Nephrology

### ★ Simulation Excellence

Dr. Gidwani — Director of Simulation

10 · Total Awards

10 · Faculty Recognized

6 · Citations Addressed

100% · Faculty Engagement

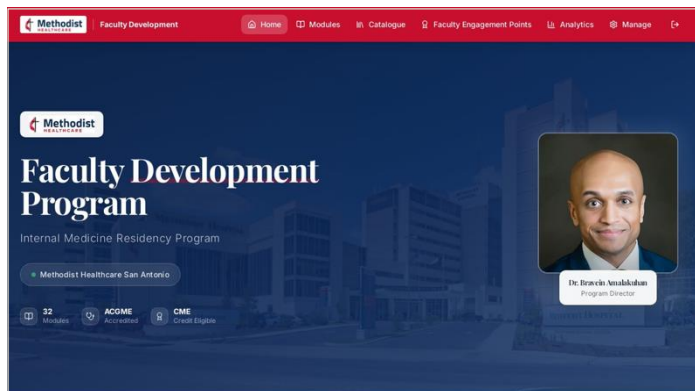
Citation #2 · Fully Remediated 

## Digital Resources & National Standing

- ▶ The Methodist GME Handbook Collection — 4 Specialty Handbooks
- ▶ CVICU · ICU · Inpatient Medicine · Outpatient Medicine — iOS & Android
- ▶ Weekly Active Users · Live Edits · Resident Milestone Tracking
- ▶ **MethodistFacultyHub.com** — Faculty Development & Engagement Platform
- ▶ Where We Stand: TOP 5% of Community IM Programs Nationally
- ▶ Fewer than 30–40 programs out of ~700 ACGME-accredited IM programs match this profile
- ▶ 42+ Active Research Projects · RCTs · 30+ Faculty Dev Modules · \$1M+ Sim Suite

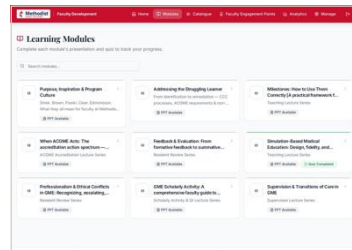
# MethodistFacultyHub.com

Education + Multi-Domain Engagement Point System | Internal Medicine Residency · AY 2025–2026



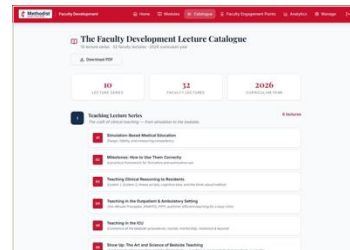
## 01 Homepage & Program Identity

Faculty Development Lectures: 32 modules, ACGME-accredited, CME-eligible.



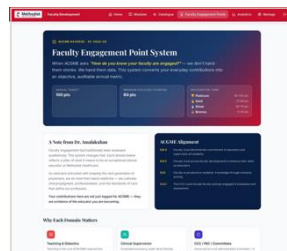
## 02 Modules Library

32 branded learning modules with integrated quizzes and progress tracking.



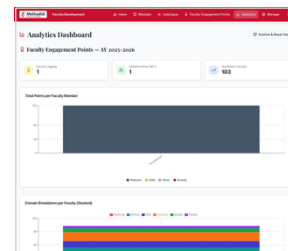
## 03 Lecture Catalogue

10 lecture series, faculty development curriculum, 2026 academic year.



## 04 Engagement Points

ACGME-aligned faculty engagement metric — objective, auditable, tiered (Platinum / Gold / Silver / Bronze). Log Page



## 05 Analytics Dashboard

Real-time faculty engagement analytics — domain breakdown, tier distribution, trends.

## Curated Evidence Library

- Cloud-based reference manager used to centralize major articles, guidelines & landmark trials.
- Organized by **19 organ-system / topic folders** — from Cardiology to Palliative Care.
- Residents access the **latest evidence on-demand** during rounds, consults & board prep.
- Continuously updated as new guidelines are released (ADA, AHA/ACC, IDSA, etc.).

19

TOPIC LIBRARIES

Always current

Shared group library: **Methodist\_GME**

zotero

Web Library Groups Documentation Forums Get Involved Bravein

Q Title, Creator, Year Upgrade Storage

My Library

My Publications

Trash

Group Libraries

Methodist\_GME

Ambulatory

Cardiac Care Unit

Cardiology

Critical Care

Didactic Lectures

Emergency Med

Endocrinology

Gastroenterology

Geriatrics

Heme/Onc

Infectious Disease

Journal Club

Nephrology

Neurology

Nightfloat

Palliative Care

Pulmonology

Adult Aged, 80 and over

Anti-HIV Agents Cabotegravir

Cardiovascular Diseases Female

Healthy Lifestyle HIV Infections

Human immunodeficiency virus Humans

Hydroxymethylglutaryl-CoA Reductase Inh...

Hyperlipidemia Hypolipidemic Agents

Male Men Middle Aged

Practice Guidelines as Topic

Filter Tags

| Title                                                                 | Creator                       | Date       |
|-----------------------------------------------------------------------|-------------------------------|------------|
| 10. Cardiovascular Disease and Risk Management: Standards of ...      |                               |            |
| 11. Chronic Kidney Disease and Risk Management: Standards of ...      |                               |            |
| 12. Retinopathy, Neuropathy, and Foot Care: Standards of Care in ...  |                               |            |
| 13. Older Adults: Standards of Care in Diabetes—2026                  |                               |            |
| 2. Diagnosis and Classification of Diabetes: Standards of Care in ... |                               |            |
| 2024 AHA/ACC/AACVPA/ASNC/HRS/SCA/SCCT/SCMR/SVM Guideline ...          | Thompson et al.               | 2024-11-05 |
| 2025 AHA/ACC/AACVPA/ABCA/ACC/AHA/ACCP/AAGS/AMA/ASPC/N...              | Jones et al.                  | 2025       |
| 2026 ACC/AHA/ACVPR/ABCA/ACC/AHA/ACCP/AAGS/AMA/ASPC/NLAP...            | Blumenthal et al.             | 2026       |
| 3. Comprehensive Medical Evaluation and Assessment of Comorb...       | American Diabetes Association | 2017-01-01 |
| 6. Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises: Sta...     |                               |            |
| 8. Obesity and Weight Management for the Prevention and Treatme...    |                               |            |
| 9. Pharmacologic Approaches to Glycemic Treatment                     | American Diabetes Association | 2017-01-01 |
| Addressing Alcohol Use                                                | Krist and Bradley             | 2025       |
| American Association of Clinical Endocrinology Consensus State...     | Patel et al.                  | 2025       |
| American Association of Clinical Endocrinology Consensus State...     | Nadolsky et al.               | 2025       |
| Anxiety Disorders: A Review                                           | Souhany and Simon             | 2022-12-27 |
| Asthma in Adults                                                      | Mosnaim                       | 2023-09-14 |
| Bipolar Disorder                                                      | Bauer                         | 2022       |
| Breast Cancer Screening and Prevention                                | Farkas and Nattinger          | 2023       |
| Care of the Patient After Metabolic and Bariatric Surgery             | Bramante et al.               | 2022       |
| Chronic kidney disease                                                | Kalantar-Zadeh et al.         | 2021-08-28 |
| Chronic Obstructive Pulmonary Disease                                 | Labaki and Rosenberg          | 2020-08-04 |
| Clinical Management of Obesity                                        | Apovian et al.                | 2025       |
| Clinical Practice Guideline for the Pharmacologic Treatment of Ch...  | Sateia et al.                 | 2017-02-15 |
| Depression                                                            | McCarron et al.               | 2021       |
| Diagnosis and Management of Gastroesophageal Reflux Disease: ...      | Phillips et al.               | 2025       |
| Dyslipidemia                                                          | Arvanitis and Lowenstein      | 2023       |
| Genitourinary Syndrome of Menopause: Management Strategies f...       | Faubion et al.                | 2017       |
| HIV Pre-Exposure Prophylaxis                                          | Liegeon et al.                | 2024       |
| HIV Pre-Exposure Prophylaxis                                          | Liegeon et al.                | 2024-09    |
| Hormone Therapy for Postmenopausal Women                              | Pinkerton                     | 2020-01-30 |

52 Items in this view

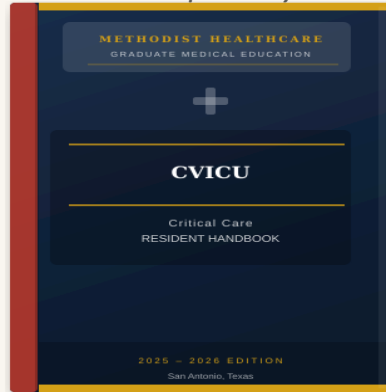
**Methodist\_GME Group Library** — organ-system folders (Cardiology, Endo, Nephro, Neuro, Pulm, Heme/Onc, ID, GI, Geri, Palliative, etc.).

zotero.org · Methodist\_GME



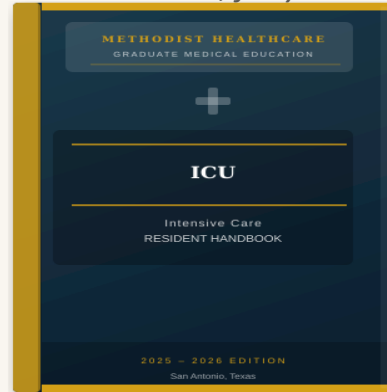
# The Methodist GME Handbook Collection

*Four specialty handbooks — built here, for your training here · Evidence-based · Updated Continuously*



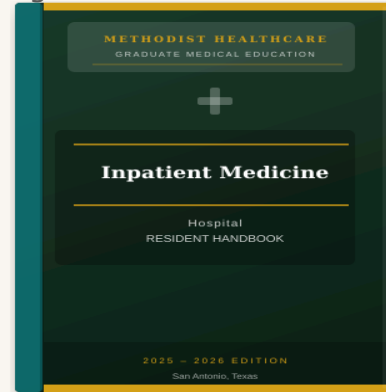
## CVICU · Handbook

*Post-op cardiac surgery · MCS  
· Hemodynamics · Shock*



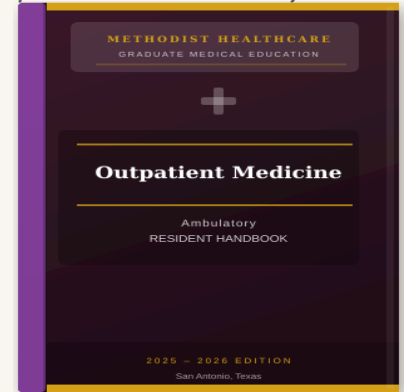
## ICU · Handbook

*Ventilator management ·  
Sepsis bundles · ARDS  
protocols*



## Inpatient · Medicine

*Hospital medicine · Common  
admissions · Rapid reference*



## Outpatient · Medicine

*NIMA clinic · Preventive care ·  
Chronic disease management*

**iOS & Android App** · weekly active users · Live edits, no reprinting ever · Resident milestone tracking · Push notifications

**Methodist Healthcare GME · Evidence-Based · Updated Continuously**

# Where We Stand: Transformative in < 1 Year

*This profile describes fewer than 30–40 programs out of ~630 ACGME-accredited IM programs nationally.*

**42+**

## Active Research Projects

<10/yr avg for community programs. Methodist competes with major academic centers.

**RCTs**

## Randomized Controlled Trials

Running RCTs as a community program is almost unheard of nationally.

**32+**

## Faculty Dev Modules

Most community programs have zero. 30+ modules with a digital library.

**\$1M+**

## Simulation Suite

Fewer than 8% of community IM programs have a fully equipped sim center.

**200+**

## Seat Auditorium

Dedicated academic space signals hospital investment few community GMEs receive.

**#2**

## Texas ACP Jeopardy

2nd in Texas at ACP Medical Jeopardy — direct measure of resident knowledge depth.

**4**

## Personalized Apps & Handbooks

Custom GME apps + handbooks on a digital platform. Frontier-level ed tech.

**#1**

## Largest Hospital in Texas

Quaternary referral center. Residents see case complexity that rivals any program.

**TOP 5% of Community IM Programs Nationally** · Fewer than 30–40 programs out of ~630 ACGME-accredited IM programs match this profile

## SECTION 06 · FORWARD PLANNING

# Forward Planning & Action Plan 2025–2026

- ▶ SWOT Analysis: Strengths & Opportunities Going Forward
- ▶ 2025–2026 Action Plan — 7 Priority Areas
- ▶ Site Visit Readiness: September 2026 — What Residents Will Say
- ▶ ITE Score Improvement Target: PGY-1 55%+ by 2026
- ▶ Graduating Class of 2026: Board Prep & Fellowship Advising
- ▶ Fellowship Pipeline: Cardiology (2027) · PCCM (2028) · GI (2028)

## 2025–2026 SWOT Analysis — Strengths & Weaknesses

### STRENGTHS

- Mission-driven leadership committed to GME excellence (1 PD, 3 APD's)
- 32-module Faculty Development Curriculum — among HCA's most comprehensive
- Strong institutional support from DIO and HCA GME infrastructure
- San Antonio majority-minority population (63%+ Hispanic/Latino) drives rich SDOH exposure
- Three dedicated SDOH modules — ahead of most IM programs nationally
- CCC fully operational with milestone-anchored documentation
- Engaged, diverse resident cohort with progressive milestone achievement
- SBME framework launched; faculty trained in simulation design and debriefing
- 40+ active GME research projects with resident involvement

### WEAKNESSES

- Faculty development recent — sustained bedside behavior change needs AY 2025–26 monitoring
- 2025 PGY-1 avg 52.6% (22nd %ile) — below national mean; Neurology and Pulm/CC gaps
- 2025 PGY-2 avg 65.7% (45th %ile) — improved vs. PGY-1 but below 50th %ile target
- Scholarly activity infrastructure newly established — weakness in 2024, turned strength mid-2025
- Limited core faculty (5, excl. PD/APDs) — expansion in progress adds admin burden
- Community faculty evaluation compliance needs ongoing follow-up
- No graduates yet (first class June 2026) — board pass and fellowship match pending
- New Innovations platform utilization needs optimization across rotations

## 2025–2026 SWOT Analysis — Opportunities & Threats

### OPPORTUNITIES

- September 2026 site visit — demonstrate complete program transformation from cited program to model program
- San Antonio's SDOH-burden population positions Methodist IM as a national health equity GME leader
- Growing national attention to SDOH in GME — program is ahead of peers in integration
- HCA system resources and GME infrastructure support faculty development at scale
- First graduating class (2026) — establish strong board pass rate and fellowship match precedent
- Outpatient expansion (NIMA Clinic) — addresses ACGME ambulatory requirements
- Faculty recruitment — hire faculty aligned with program values from day one
- Social media/open house pipeline strengthening applicant quality and diversity

### THREATS

- Active citation status with Warning — any compliance gap before September 2026 carries significant risk
- Federal policy shifts re: Health Equity & Inclusion in medical education create regulatory uncertainty for SDOH programming
- Faculty retention — newly trained faculty are high-value targets for other programs
- Texas Medicaid non-expansion and high uninsured rate create financial strain affecting GME resources
- ACGME resident and faculty surveys (Feb–Apr 2026) directly input to site visitor assessment
- Burnout and wellness risks elevated for both residents and PD during citation remediation year
- Competitive IM programs in San Antonio (UTHealth San Antonio and others) for applicant recruitment
- Community faculty evaluation compliance creates quality control vulnerability

## 2026–2027 SWOT — Strengths & Opportunities Going Forward

### STRENGTHS GOING INTO 2026–2027

- 32-module faculty development library delivered via app — biweekly cadence, analytics-tracked accountability
- PGY-2 cohort (n=9) performing at/above national mean — 7 of 9 improved scores year-over-year
- PGY-3 residents (Melo, Wright) on track for strong board performance — June 2026 graduation
- Morning Report, Noon Conference, and Monthly Expert Board Review — 3-tier daily/monthly learning scaffold
- Expert board reviewer scored 275 USMLE Step 3 — highest-caliber board preparation content
- Four resident educational handbooks — CVICU, ICU, Inpatient, Outpatient — ABIM-grounded
- CCC fully operational with milestone narrative templates and on-time ADS submission
- SDOH integrated into curriculum — ahead of most IM programs nationally
- Site visit preparation is active, documented, and evidence-rich · New Education Center (6,400 sq ft, 232-seat auditorium) opens 7/27/26 — major institutional investment in GME infrastructure

### OPPORTUNITIES FOR 2026–2027

- September 2026 site visit — demonstrate full transformation from cited program to model program
- First board-eligible class — establish strong pass rate precedent for program reputation
- ACGME survey results (Apr 2026) — opportunity to show measurable improvement from prior year
- PGY-1 2025 class (n=14) — large cohort, multiple high-potential residents to develop
- Continued expansion of outpatient training sites — Methodist Health Ministries operational · Fellowship pipeline on track: Cardiology (2027), PCCM (2028), GI (2028)
- Faculty recruitment — bring in faculty already aligned with program values and culture
- QI and scholarly activity infrastructure in place — enable resident and faculty productivity
- Social media/open house pipeline strengthening — build applicant pool quality and diversity

# 2026–2027 Action Plan

Priorities for the year ahead — site visit preparation, ITE improvement, sustained growth

| PRIORITY AREA                   | ACTIONS                                                                                                                                                      | RESPONSIBLE               | TARGET       | STATUS       |
|---------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------|--------------|--------------|
| Site Visit Prep<br>(Sept 2026)  | Complete citation response docs · evidence portfolio · Faculty Leadership Domain restructuring                                                               | PD + Core Faculty         | Aug 2026     | COMPLETED    |
| ITE Score Improvement           | PGY-1 2025 avg 52.6% (22nd %ile) → target 55%+ by June 2026 · focused board review                                                                           | PD + Faculty              | Jun 2026     | COMPLETED    |
| ACGME Survey Response           | Analyze April 2026 survey results immediately after release. Address any areas scoring below national mean with targeted interventions before site visit.    | PD + Myriam Gandy         | May–Jun 2026 | PENDING DATA |
| Scholarly Activity Output       | Each resident completes min. 1 documented scholarly activity. QI projects submitted and presented. Journal club monthly. Chief residents maintain oversight. | Chief Residents + Faculty | Jun 2026     | COMPLETED    |
| Graduating Class<br>(June 2026) | Board review, fellowship advising, graduation requirements verification. PGY-3 milestone targets: Level 3–4 across all competency domains.                   | PD + Program Admin        | Jun 2026     | COMPLETED    |
| Faculty Development             | Complete remaining faculty dev modules · Matrix governance sustains engagement                                                                               | Program Director          | Jun 2026     | COMPLETED    |
| New Innovations Optimization    | Full utilization of evaluation, milestone, and duty hour modules across all rotations. Community faculty compliance rate tracked and improved.               | Program Admin + PD        | Jun 2026     | COMPLETED    |
| Outpatient Curriculum           | Continued Accreditation — prerequisite for all fellowship applications                                                                                       | PD + Dr. Triana/Melo      | Jun 2026     | COMPLETED    |

# Board Exam Preparation & Fellowship Readiness — 2026-28

First graduating class: June 2026 | Active preparation underway | Infrastructure built to maximize pass rates and fellowship outcomes

## BOARD EXAM PREPARATION SYSTEM

**First class graduates June 2026 — board exam results reported in AY 2025–2026 APE**

Monthly expert board review led by faculty with USMLE Step 3 score of >275 — highest-caliber board prep content. MKSAP 2025–2026 distributed to all residents. ITE weak area analysis (Neurology, Pulm/CC, Gen IM) directly drives noon conference curriculum adjustments. PGY-2 avg 65.7% (45th %ile) — 7 of 9 improved raw scores year-over-year. PGY-3s on track for Level 3–4 milestones at graduation.

## FELLOWSHIP ADVISING SYSTEM

**First fellowship match cycle: March 2027 — infrastructure fully operational**

PD conducting individualized career advising sessions with all PGY-2/3 residents. Fellowship interest survey completed — specialty distribution mapped. Letters of recommendation process established and documented. Fellowship pipeline supported by Cardiovascular Fellowship (2027), PCCM (2028), and GI (2028) — MHS becoming a regional training hub that strengthens match competitiveness.

## PREPARATION FOR CLASS OF 2026

### Board Review

Monthly expert board review (faculty: 275 Step 3) · MKSAP distributed to all residents · ITE gap analysis → noon conference adjustments targeting Neurology, Pulm/CC, Gen IM · PGY-1 target: 55%+ by 2026 ITE · Morning Report daily case-based reasoning reinforcement

### Clinical Competency

CCC June 2025: all graduating residents on trajectory for Level 3–4 across PC, MK, SBP, PBLI, PROF, ICS · CCC Dec 2025 (PGY-1 semi-annual) completed with full quorum · Milestone narrative templates deployed with behavioral descriptor language

### Fellowship Advising

Individual career advising sessions: PD meets 1:1 with all PGY-2/3 residents · Specialty interest survey mapped to subspecialty exposure · LoR process established in NI · Elective expansion , adds subspecialty depth for competitive applications

### Graduation Requirements

Graduation policy reviewed and distributed to all residents · All requirements documented in NI · PGY-3s (Wright, Melo S.) tracking on schedule for June 2026 graduation · Board exam registration guidance issued



# Thank You

*To Our Team — For Building This in Less Than a Year*

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**32**

Faculty Modules

**19**

Topic Libraries · 100+ Articles

**Apps +**

Personalized Digital Platforms

**4-Tier**

Engagement System

## WHAT WE BUILT TOGETHER

[methodistsaim.com](#) · [MethodistFacultyHub.com](#) · [Zotero Evidence Library](#)

**4 Comprehensive Handbooks** · **Leading Quality Metrics** · **Curriculum Transformation**

Active Research & RCTs · 2nd at TX ACP Jeopardy · Simulation & didactic infrastructure

*Structure · Programmatic Growth · Evidence & Feedback-Driven Improvement*

NEXT: ACGME SITE VISIT · SEPTEMBER 2026